



Change Management for successful AVT adoption



This toolkit is for NHS organisations and teams preparing to introduce or are planning to scale up Ambient Voice Technology (AVT). It is designed to support those responsible for leading or supporting change, particularly where there may be limited experience of formal change management.



For successful Digital Adoption you need



PEOPLE

Engage and support the people who will use and be impacted by AVT



PROCESS

Adapt workflows and ways of working for better outcomes



TECHNOLOGY

Use technology effectively to enable and enhance care



Why is this important?

This toolkit provides a clear, practical approach to managing the people side of the change, helping teams build confidence, reduce resistance and support successful and sustained adoption of AVT in practice.

Change Management provides the structure to help you understand



What is changing

Increase awareness and clarity



Why it matters

Understand the benefits and the impact



How it affects your day-to-day work

Support new ways of working



This toolkit is not about how to implement AVT

This Toolkit complements the AVT Adoption Playbook. The Playbook provides the end-to-end deployment approach for AVT, while this Toolkit provides detailed change management guidance, tools, templates and checklists to support successful adoption.



Our change in action

Working together to make AVT a positive, everyday part of care and clinical practice



Adoption of Ambient Voice Technology



Digital adoption doesn't happen on its own, it needs **deliberate change management**, using practical tools and techniques to support people through the change.



AVT isn't just a technical rollout, it changes how people work on a day-to-day basis and is about workflow redesign. Change management focusses on supporting people to feel confident and supported to use the new technology in practice and adopt new ways of working to increase the likelihood AVT is adopted, used and sustained.



Feel confident
People feel capable using AVT in practice



Feel supported
People have the right information, training and resources



Adopt new ways of working
Workflows are simpler, safer and more efficient



Increase likelihood
AVT is adopted, used and sustained



Understand the change



Communicate and engage



Support and build confidence



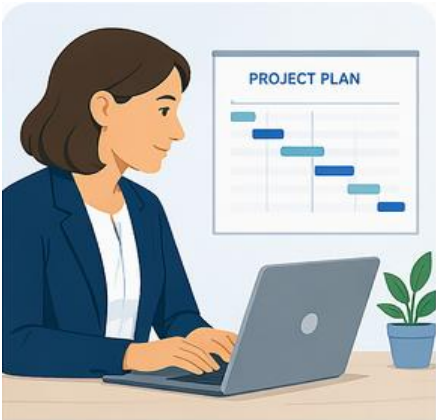
Adopt, use and sustain

This toolkit has been designed to allow organisations to understand the change activities based on their current position; it's not one size fits all.



Who this Toolkit is for

This toolkit is designed for people delivering Ambient Voice Technology (AVT) solutions into their organisation



Programme / Project Managers

Plan, deliver and oversee successful AVT adoption.



Change Managers

Lead the people side of change and build confidence.



Transformation and Operational Leads

Drive alignment and embed AVT in ways of working.



Clinical and Administrative Leads

Design and champion AVT in practice, strengthening adoptions across teams.



Digital / Informatics Leads

Ensure the technology, data and systems enable safe and effective use.



You do not need to be a change management expert to use this toolkit.

Users should work through the AVT Adoption Playbook alongside this Toolkit. The Playbook explains what activities need to be completed; this Toolkit explains how to deliver them effectively.

How to navigate this toolkit



Click on this icon to return to the start



Click on this icon to get to the next page



Click on this icon to go back a page



Click on this icon to return to the Pathway Map



Click on this icon to return to Pathway Activities

Pathway Map

3 AVT pathways to follow based on your organisation's current position.

Activity Maps

Detailed maps showing the activities in each stage.

Roadmaps

Guidance for what good looks like at each stage.

Interactive toolkit

Look for clickable items throughout the toolkit. Select items to go directly to the resource.

Checklist links

Open the relevant checklists for your chosen pathway.

Resources and Tools

Access supporting materials for your AVT project and the full [Change Management Toolkit](#) for further advice and resources.

Tip: All Pathway boxes, Maps, 'Are you on track?', Checklists and Resources are interactive. Simply select the item to continue to that content.

Use the AVT Adoption Playbook to understand:

- What activities need completing
- When they should be completed
- Deployment readiness criteria

- Use this Toolkit to access supporting guidance, checklists and tools.



AVT Digital Adoption Toolkit Pathway Map:

Before selecting a pathway, review the AVT Adoption Playbook to understand the deployment lifecycle and identify your current implementation stage

Where are you right now?

We are starting AVT for the first time

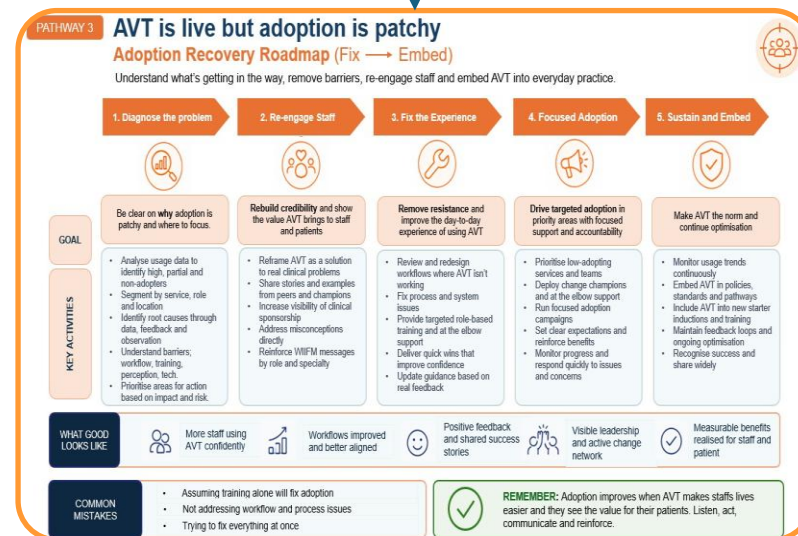
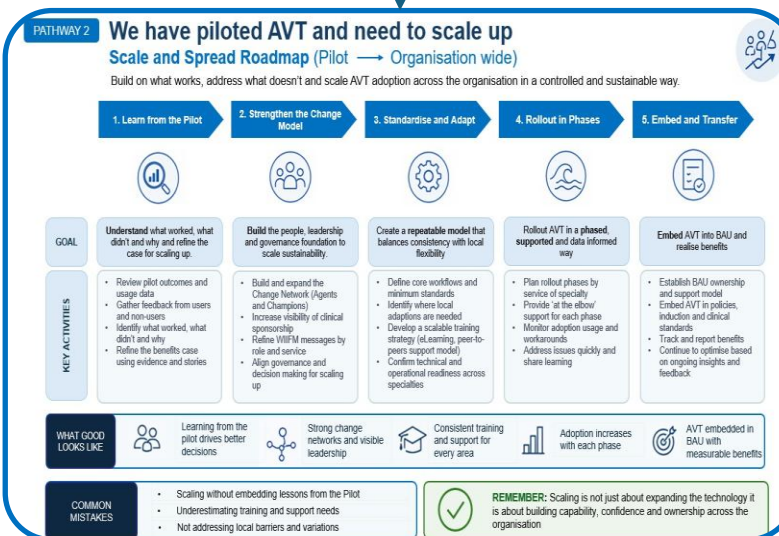
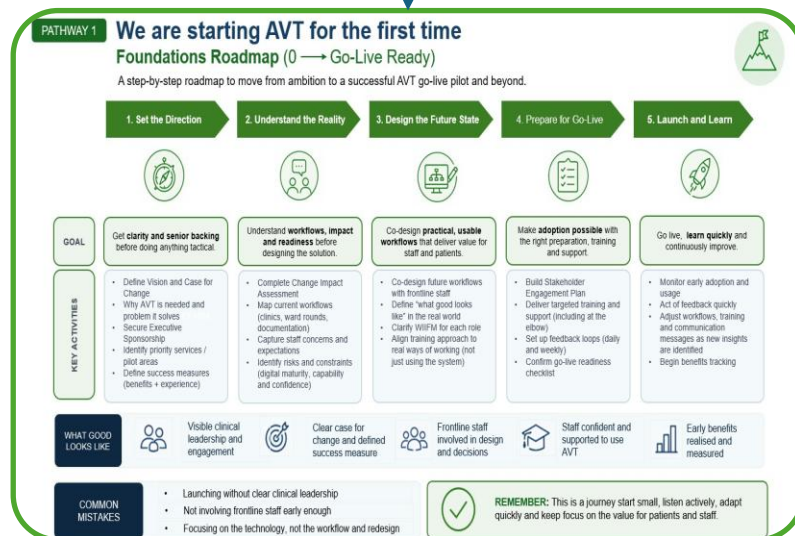
Start with – Pathway 1

We have piloted AVT and need to scale up

Scale with – Pathway 2

AVT is live but adoption is patchy

Focus on – Pathway 3



Please select the image of the required pathway to continue



We are starting AVT for the first time

Foundations Roadmap (0 → Go-Live Ready)

A step-by-step roadmap to move from ambition to a successful AVT go-live pilot and beyond.



1. Set the Direction



GOAL

Get **clarity and senior backing** before doing anything tactical.

KEY ACTIVITIES

- Define the Vision and Case for Change
- Define why AVT is needed and the problem it solves
- Secure Executive Sponsorship
- Identify priority services / pilot areas
- Define success measures (benefits + experience)

2. Understand the Reality



Understand **workflows, impact and readiness** before designing the solution.

- Complete the Change Impact Assessment
- Map current workflows (clinics, ward rounds, documentation)
- Capture staff concerns and expectations
- Identify risks and constraints (digital maturity, capability and confidence)

3. Design the Future State



Co-design **practical, usable workflows** that deliver value for staff and patients.

- Co-design future workflows with frontline staff
- Define “what good looks like” in the real world
- Clarify What’s In It For Me (WIIFM) Benefits for each role
- Align training approach to real ways of working (not just using the system)

4. Prepare for Go-Live



Make **adoption possible** with the right preparation, training and support.

- Build Stakeholder Engagement Plan
- Deliver targeted training and support (including “at the elbow”)
- Set up feedback loops (daily and weekly)
- Confirm go-live readiness checklist

5. Launch and Learn



Go live, **learn quickly** and continuously improve.

- Monitor early adoption and usage
- Act on feedback quickly
- Adjust workflows, training and communication messages as new insights are identified
- Begin benefits tracking

WHAT GOOD LOOKS LIKE



Visible clinical leadership and engagement



Clear case for change and defined success measure



Frontline staff involved in design and decisions



Staff confident and supported to use AVT



Early benefits realised and measured

COMMON MISTAKES

- Launching without clear clinical leadership
- Not involving frontline staff early enough
- Focusing on the technology, not the workflow and redesign



REMEMBER: This is a journey; start small, listen actively, adapt quickly and keep focus on the value for patients and staff.

THIS ROADMAP SUPPORTS THE DEPLOYMENT ACTIVITIES DESCRIBED IN THE [AVT ADOPTION PLAYBOOK](#).

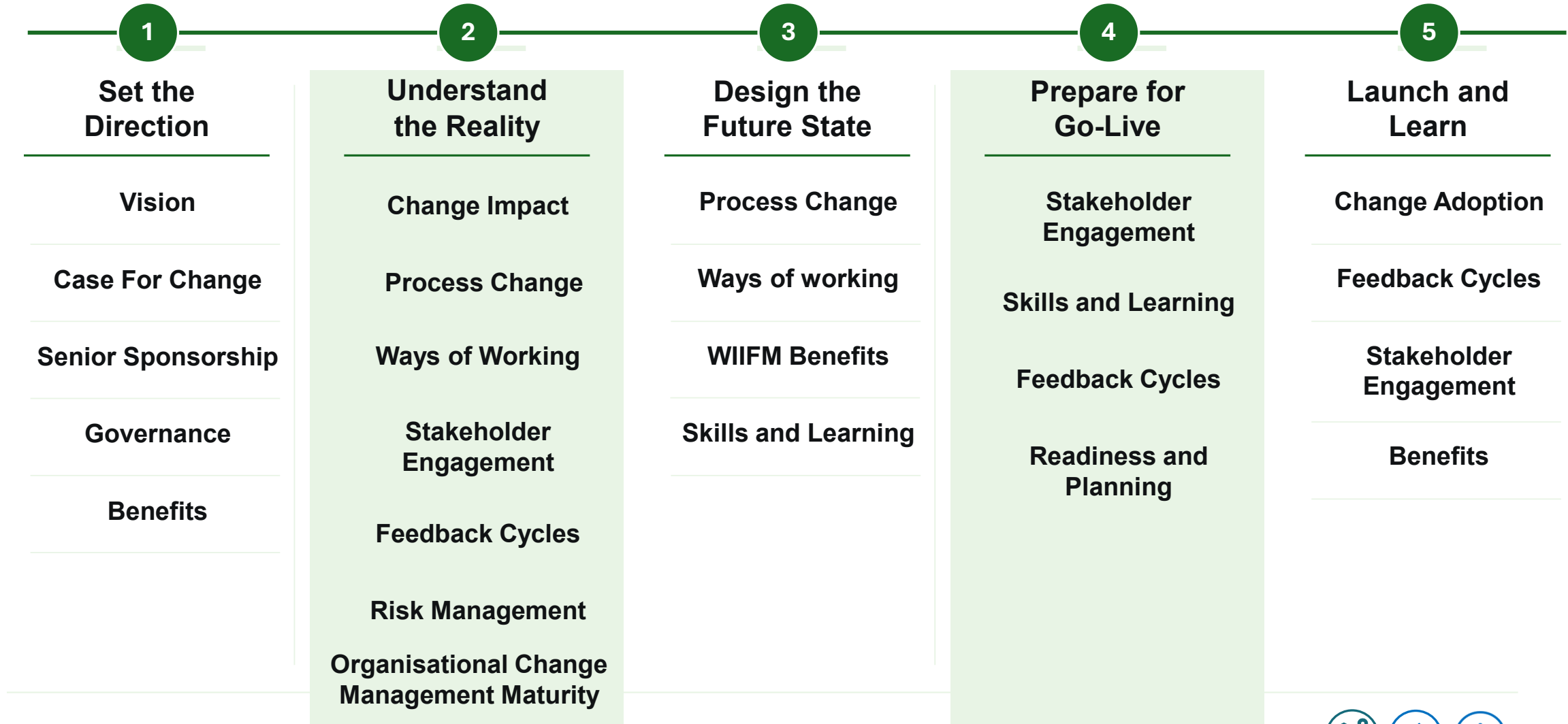


We are starting AVT for the first time



Foundations Roadmap (0 → Go-Live Ready)

Select the appropriate activity below for further information and supporting checklist



We have piloted AVT and need to scale up

Scale and Spread Roadmap (Pilot → Organisation wide)



Build on what works, address what doesn't and scale AVT adoption across the organisation in a controlled and sustainable way.

1. Learn from the Pilot



GOAL

Understand what worked, what didn't and why and refine the case for scaling up.

KEY ACTIVITIES

- Review pilot outcomes and usage data
- Gather feedback from users and non-users
- Identify what worked, what didn't and why
- Refine the benefits case using evidence and stories

2. Strengthen the Change Model



Build the people, leadership and governance foundation to scale sustainability.

- Build and expand the Change Network (Agents and Champions)
- Increase visibility of clinical sponsorship
- Refine WIIFM benefits messages by role and service
- Align governance and decision making for scaling up

3. Standardise and Adapt



Create a **repeatable model** that balances consistency with local flexibility

- Define core workflows and minimum standards
- Identify where local adaptations are needed
- Develop a scalable training strategy (eLearning, peer-to-peers support model)
- Confirm technical and operational readiness across specialties

4. Rollout in Phases



Rollout AVT in a **phased, supported** and data informed way

- Plan rollout phases by service or specialty
- Provide "at the elbow" support for each phase
- Monitor adoption usage and workarounds
- Address issues quickly and share learning

5. Embed and Transfer



Embed AVT into Business as Usual (BAU) and realise benefits

- Establish BAU ownership and support model
- Embed AVT in policies, induction and clinical standards
- Track and report benefits
- Continue to optimise, based on ongoing insights and feedback

WHAT GOOD LOOKS LIKE



Learning from the pilot drives better decisions



Strong change networks and visible leadership



Consistent training and support for every area



Adoption increases with each phase



AVT embedded in BAU with measurable benefits

COMMON MISTAKES

- Scaling without embedding lessons from the Pilot
- Underestimating training and support needs
- Not addressing local barriers and variations



REMEMBER: Scaling is not just about expanding the technology, it is about building capability, confidence and ownership across the organisation

THIS ROADMAP SUPPORTS THE DEPLOYMENT ACTIVITIES DESCRIBED IN THE [AVT ADOPTION PLAYBOOK](#).





Scale and Spread Roadmap (Pilot → Organisation wide)

Select the appropriate activity below for further information and supporting checklist

1

Learn from the Pilot

- Capability and Confidence
- Feedback Cycles
- Resistance Management
- Reinforcement
- Benefits
- WIIFM Benefits

2

Strengthen the Change Model

- Change Network
- Senior Sponsorship
- WIIFM Benefits
- Governance
- Organisational Change Management Maturity

3

Standardise and Adapt

- Process Change
- Ways of Working
- Change Adoption
- Readiness and Planning

4

Rollout in Phases

- Readiness and Planning
- Skills and Learning
- Change Adoption
- Resistance Management
- Ways of Working
- Reinforcement
- Feedback Cycles

5

Embed and Transfer

- Transfer to BAU
- Ways of Working
- Benefits
- Feedback Cycles
- Reinforcement



AVT is live but adoption is patchy

Adoption Recovery Roadmap (Fix → Embed)

Understand what's getting in the way, remove barriers, re-engage staff and embed AVT into everyday practice.



1. Diagnose the problem



Be clear on **why** adoption is patchy and where to focus.

- Analyse usage data to identify high, partial and non-adopters
- Segment by service, role and location
- Identify root causes through data, feedback and observation
- Understand barriers; workflow, training, perception, technology.
- Prioritise areas for action based on impact and risk.

2. Re-engage Staff



Rebuild credibility and show the value AVT brings to staff and patients

- Reframe AVT as a solution to real clinical problems
- Share stories and examples from peers and champions
- Increase visibility of clinical sponsorship
- Address misconceptions directly
- Reinforce WIIFM messages by role and specialty

3. Fix the Experience



Remove resistance and improve the day-to-day experience of using AVT

- Review and redesign workflows where AVT isn't working
- Fix process and system issues
- Provide targeted role-based training and "at the elbow" support
- Deliver quick wins that improve confidence
- Update guidance based on real feedback

4. Focused Adoption



Drive targeted adoption in priority areas with focused support and accountability

- Prioritise low-adopting services and teams
- Deploy change champions and "at the elbow" support
- Run focused adoption campaigns
- Set clear expectations and reinforce benefits
- Monitor progress and respond quickly to issues and concerns

5. Sustain and Embed



Make AVT the norm and continue optimisation

- Monitor usage trends continuously
- Embed AVT in policies, standards and pathways
- Include AVT into new starter inductions and training
- Maintain feedback loops and ongoing optimisation
- Recognise success and share widely

WHAT GOOD LOOKS LIKE



More staff using AVT confidently



Workflows improved and better aligned



Positive feedback and shared success stories



Visible leadership and active change network



Measurable benefits realised for staff and patient

COMMON MISTAKES

- Assuming training alone will fix adoption
- Not addressing workflow and process issues
- Trying to fix everything at once



REMEMBER: Adoption improves when AVT makes staffs lives easier and they see the value for themselves and their patients. Listen, act, communicate and reinforce.

THIS ROADMAP SUPPORTS THE DEPLOYMENT ACTIVITIES DESCRIBED IN THE [AVT ADOPTION PLAYBOOK](#).





Select the appropriate activity below for further information and supporting checklist

1

Diagnose the problem

Change Adoption

Feedback Cycles

Change Impact

Risk Management

Organisational Change
Management Maturity

2

Re-engage Staff

Vision

Case for Change

Change Network

Stakeholder
Engagement

WIIFM Benefits

3

Fix the Experience

Ways of Working

Process Change

Skills and Learning

Reinforcement

Feedback Cycles

4

Focused Adoption

Change Adoption

Change Network

Stakeholder
Engagement

Vision

Case for Change

Feedback Cycles

5

Sustain and Embed

Change Adoption

Ways of Working

Benefits

Feedback Cycles

Celebrating Success



Vision for Change

Set the direction before you design the solution

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Describe the vision and future



PATHWAY 2 – We have piloted AVT and need to scale up

Align teams around shared vision



PATHWAY 3 – AVT is live, but adoption is patchy

Reset the vision and intended purpose



WHAT IT IS

Vision for Change sets out why AVT matters, how it will improve patient care and staff experience and what good looks like in everyday clinical and administrative work.



USER INSIGHT



AVT landed best when it was framed as a clinical workflow and behaviour change, not a technology rollout.



WHY THIS MATTERS



Reduces confusion about what AVT is (and isn't) and protects trust in the change early.



Prevents the programme being judged only on technology rather than the intended workflow improvement.



Provides a consistent narrative across clinical, operational and admin audiences, reducing rumour-driven resistance.



QUICK REALITY CHECK

Can staff describe AVT in one sentence?

Does it reflect real pressures, not just programme goals?

Are leaders saying the same thing consistently?

Do people understand what will change in their day-to-day work?





Vision for Change

What good looks like – and what's at risk if we don't get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Vision checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Vision checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Vision checklist



WHAT GOOD LOOKS LIKE



CLEAR AND COMPELLING

Vision is simple, inspiring and focused on patient care, staff experience and better ways of working.



UNDERSTOOD AND SHARED

Staff at all levels can explain in their own words why AVT matters and what will change.



CONSISTENTLY COMMUNICATED

Leaders reinforce the vision regularly and use it to guide decisions and priorities.



GROUNDING IN REAL SITUATIONS

Vision reflects the day-to-day challenges teams face and the improvements AVT will bring.



GUIDES ACTIONS

Vision helps teams make choices, solve problems and stay focused on what matters.



RISKS IF YOU DON'T DO THIS



SEEN AS A TECHNOLOGY ROLLOUT

Focus stays on the tool rather than the workflow and behaviour change.



CONFUSION AND MIXED MESSAGES

Different teams hear different things, leading to uncertainty about the reason AVT is being introduced.



LOW ENGAGEMENT AND RESISTANCE

Staff don't see the relevance to their day-to-day work and are less likely to adopt AVT.



INCONSISTENT ADOPTION

Without a shared vision, use will vary widely across teams and services.



BENEFITS NOT REALISED

Without a clear direction, efforts drift and the value of AVT is not achieved.



TOP TIPS

TOP TIP 1 Start with the 'why'

TOP TIP 2 Show the future clearly

TOP TIP 3 Make it memorable



LINKS AND TOOLS

Vision for Change
Resource

NHS Futures Change
Management Toolkit
Tool





Case for Change

AVT must solve a problem that staff clearly recognise

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Explain the problem and need



PATHWAY 2 – We have piloted AVT and need to scale up

Strengthen evidence to support the case



PATHWAY 3 – AVT is live, but adoption is patchy

Reconnect case to real-world pressures



WHAT IT IS

A Case for Change explains the real clinical and operational pressures AVT is responding to, why change is needed now and why current ways of working are becoming unsustainable.



USER INSIGHT

“ AVT was most effective when it clearly addressed real operational pressures such as backlogs, wait times and admin burdens. ”



WHY THIS MATTERS



Prevents confusion over whether AVT is about productivity, wellbeing, backlog reduction or system replacement.



Helps teams agree on the workflow changes needed to realise the benefits.



Reduces early resistance by focusing on the problem that needs solving, not the novelty of AVT.



QUICK REALITY CHECK

Can we explain why change is needed now?

Are the benefits specific and measurable?

Can leaders explain the change in one simple story?

Is the problem clearly defined or are you starting with AVT as the answer?





Case for Change

What good looks like – and what’s at risk if we don’t get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Case for Change checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Case for Change checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Case for Change checklist



WHAT GOOD LOOKS LIKE



AVT NOT SEEN AS A TECHNICAL ROLLOUT

AVT is consistently understood as a clinical workflow and behaviour change, not a technology rollout.



LOWER COGNITIVE LOAD

Clinicians experience lower cognitive load, better patient interaction and stronger day-to-day usability.



RESISTANCE REDUCTION

Reduces early resistance by focusing on the problem that needs solving, not the novelty of AVT.



CLEAR PROBLEM STATEMENT

The problem AVT is solving is specific, evidence-based and easy to explain.



COMPELLING REASON TO ACT NOW

There is urgency, not just a general desire to improve.



RISKS IF YOU DON'T DO THIS



STALLED ADOPTION

Adoption stalls once the novelty fades because the purpose was never clear.



DISENGAGEMENT

Productivity narratives trigger suspicion and disengagement in AVTs introduction.



SCALING JUSTIFICATION STRUGGLES

Leaders struggle to justify scaling because there is no credible “why now” and “what problem”.



BUSINESS OUTCOMES NOT LINKED

Business Outcomes are not clearly linked to AVT.



DISAGREEMENTS REGARDING THE PROBLEM

Stakeholders do not agree on the problem statement.



TOP TIPS

TOP TIP 1 Start with the problem you are trying to solve

TOP TIP 2 Quantify the value and the risk of doing nothing

TOP TIP 3 Create one simple change story



LINKS AND TOOLS

Case for Change Resource

NHS Futures Change Management Toolkit





Benefits

Be clear on expected benefits, baseline early and track to evidence impact

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Define measurable benefits and improvements



PATHWAY 2 – We have piloted AVT and need to scale up

Evidence benefits using real performance data



PATHWAY 3 – AVT is live, but adoption is patchy

Re-establish benefits not being realised



WHAT IT IS

Benefits describe the practical improvements AVT is expected to deliver for clinicians, patients and services and how those improvements will be evidenced and sustained.



USER INSIGHT



Benefits landed best when they reflected real improvements in day-to-day work, not just time saved or productivity gains.



WHY THIS MATTERS



Benefits framing is a major driver of trust and motivation; it determines whether staff persist with the change.



Clear benefits reduce resistance and prevent AVT being perceived as a tool to push more patients.



Clear, identifiable benefits will support business cases and scaling decisions (not just “it feels good”).



QUICK REALITY CHECK

Are the benefits measurable in real service terms?

Do you know the starting point?

Who owns the benefits after go live?

Does AVT reduce workload or just move it?





Benefits

What good looks like – and what’s at risk if we don’t get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Benefits and WIIFM checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Benefits and WIIFM checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Benefits and WIIFM checklist



WHAT GOOD LOOKS LIKE



REAL TERM BENEFITS

Benefits are described in real terms (reduced cognitive load, reduced admin/catch-up, better patient interaction), not just minutes saved.



TAILORED MESSAGING

Benefits messaging is tailored to care setting / speciality and avoids generic productivity claims.



EVIDENCED BENEFITS

Adoption and feedback evidence is used alongside usage metrics to explain where benefits are appearing.



CLEAR OWNERSHIP

Every benefit has a clear owner and baseline.



LEADERSHIP ACCESS TO DATA

Leadership can see which benefits are on track, at risk and realised.



RISKS IF YOU DON'T DO THIS



NO BASELINING

Without baselines or a measurement plan, benefits are unverifiable and scaling decisions weaken.



OVER PROMISING

Overpromising productivity or admin reduction undermines trust and causes resistance.



BENEFITS NOT SHARED

Benefits are not widely shared. Adoption remains dependent on enthusiasts rather than evidence.



NOT LINKED TO MILESTONES

Benefits are not linked to delivery milestones, so progress on activity does not translate clearly into value.



INCONSISTENT MEASURES

Measures are inconsistent across workstreams making it hard to compare progress or aggregate value.



TOP TIPS

TOP TIP 1 Define a clear baseline

TOP TIP 2 Assign named owners

TOP TIP 3 Review benefits on a regular basis



LINKS AND TOOLS

Benefits and WIIFM Resource

NHS Futures Change Management Toolkit Tool





Senior Sponsorship

Make AVT clinically led and operationally owned, not IT driven

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Establish visible leadership and ownership



PATHWAY 2 – We have piloted AVT and need to scale up

Maintain strong and visible sponsorship



PATHWAY 3 – AVT is live, but adoption is patchy

Re-engage leaders to visibly support



WHAT IT IS

Sponsorship means visible clinical and operational senior leaders driving AVT as service improvement, supporting teams, removing barriers and building confidence to support adoption.



USER INSIGHT

“ AVT succeeded when strong clinical and operational sponsorship ensured AVT was delivered as a clinical service improvement, not an IT led change, with leaders driving adoption, behaviours and redesign. ”



WHY THIS MATTERS



Visible senior clinical and operational leadership builds trust, reduces resistance and enables pace and scale.



Ensures AVT is a clinical workflow change with clinical direction and operational ownership of local redesign.



Sponsors remove barriers, support teams and make the decisions related to risk and workflow needed to embed change.



QUICK REALITY CHECK

Are senior leaders actively showing support?

Do sponsors demonstrate behaviours they expect from others?

Are sponsors listening to feedback and responding?

Are sponsors using the same consistent story?





Senior Sponsorship

What good looks like – and what’s at risk if we don’t get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Senior Sponsorship and Governance checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Senior Sponsorship and Governance Checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Senior Sponsorship and Governance checklist



WHAT GOOD LOOKS LIKE



CLEAR SPONSORSHIP IN PLACE

Clear credible sponsorship in place – named SRO, clinical sponsor (e.g. Medical Director/CCIO) and operational lead with ownership beyond digital/IT.



VISIBLE LEADERSHIP

Visible leadership driving change on the ground – actively engaging teams, reinforcing new behaviours and removing barriers to adoption.



OPERATIONAL OWNERSHIP

Operational ownership embedded early and leading workflow redesign.



FRONTLINE FEEDBACK

Frontline feedback is heard and acted on.



REGULAR REVIEWS

Senior leaders regularly review progress, risks and benefits.



RISKS IF YOU DON'T DO THIS



FAILURE TO SCALE

Without senior clinical and operational leadership, adoption depends on individual enthusiasts and fails to scale across services.



NO HUMAN IN THE LOOP

Workflow issues and new ways of working, including “Human in the Loop” review and approval, may not be owned or resolved.



SLOW UPTAKE

Without visible senior backing, teams are less confident to change practice, delaying adoption and limiting impact.



DELAYED DECISION MAKING

Decisions are delayed because ownership is unclear.



INCONSISTENT MESSAGING

Leadership messages are inconsistent across services.



TOP TIPS

TOP TIP 1 Recruit the right sponsors early

TOP TIP 2 Ensure sponsors are visible and active

TOP TIP 3 Maintain sponsorship beyond launch



LINKS AND TOOLS

Senior Sponsorship and Governance Resource

NHS Futures Change Management Toolkit Tool





Governance

Make Governance arrangements clear so teams trust AVT and use it with confidence

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Set clear structure for decisions



PATHWAY 2 – We have piloted AVT and need to scale up

Ensure governance supports scaling effectively



PATHWAY 3 – AVT is live, but adoption is patchy

Fix gaps slowing decision-making progress



WHAT IT IS

Governance provides the structure for making clear timely decisions, so AVT is introduced safely, consistently and at scale by having the right clinical, digital and safety checks in place.



USER INSIGHT



Clear proportionate governance enabled safe timely decisions and supported AVT scaling across clinical, operational and digital teams.



WHY THIS MATTERS



Drives timely and clear decisions through defined decision making, governance cadence and active oversight.



Enables safe, compliant use by ensuring assurance (IG, DPIA, Cyber, clinical safety and consent) is in place before clinical deployment.



Supports scaling beyond pilots by managing risks transparently and creating a controlled organisation wide approach.



QUICK REALITY CHECK

Are roles clear across business, tech, legal, risk and Ops?

Who actually owns the decision, risk and sign off?

Is there a clear escalation path when something fails?

Are policies being followed in practice, not just documented?





Governance

What good looks like – and what’s at risk if we don’t get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Senior Sponsorship and Governance checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Senior Sponsorship and Governance checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Senior Sponsorship and Governance checklist



WHAT GOOD LOOKS LIKE



CLEAR GOVERNANCE

There is a clear AVT governance forum aligned to existing clinical and operational structures.



CLEAR DECISION RIGHTS

Decision rights are clear and defined (who decides and owns what across IG, IT, clinical and operational teams).



CLEAR ESCALATION ROUTES

Clear escalation routes ensure risks, behavioural challenges and adoption issues are raised and acted on.



SAFE DELIVERY

Governance enables safe delivery rather than blocking progress



DOCUMENT DECISION RIGHTS

Decision rights are documented: who can approve, pause or reject.



RISKS IF YOU DON'T DO THIS



NO GOVERNANCE

Without governance, there is no clear decision making between IG, IT and clinical/operational teams which may lead to delays.



HIDDEN RISK

Risks may remain hidden or unmanaged leading to workarounds and further risks.



TRANSITION TO SCALING UP

Without proportionate governance, pilots don’t transition into controlled organisation wide roll outs.



MONITORING

There may not be any ongoing monitoring after deployment.



RECURRING ISSUES

Issues will recur because root causes are not addressed.



TOP TIPS

TOP TIP 1 Make ownership explicit

TOP TIP 2 Review governance after launch

TOP TIP 3 Keep governance proportionate to risk



LINKS AND TOOLS

Senior Sponsorship and Governance Resource

NHS Futures Change Management Toolkit





Stakeholder Engagement and Communication

Engage early and involve staff in shaping how AVT will work

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Build awareness, trust and understanding



PATHWAY 2 – We have piloted AVT and need to scale up

Ensure messaging is clear and consistent



PATHWAY 3 – AVT is live, but adoption is patchy

Address confusion and re-engage teams



WHAT IT IS

Stakeholder Engagement and Communications build understanding, trust and commitment through early, two-way engagement with clinicians, admin teams, leaders and patients.



USER INSIGHT



Adoption was strongest where trusted peers led the change, clinical and admin teams were engaged early and patients and clinicians had clarity and confidence around the change.



WHY THIS MATTERS



Engagement reduces resistance by building understanding and addressing misconceptions early.



When people are involved in design, they become advocates.



Patient trust and understanding is increased when communication is transparent and choice based.



QUICK REALITY CHECK

Is there a feedback loop after communications go out?

Are comms understandable to non-technical staff?

Are issues tracked through to closure?

Are real life scenarios included in validations?





Stakeholder Engagement and Communication

What good looks like – and what's at risk if we don't get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Stakeholder Engagement and Communication checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Stakeholder Engagement and Communication checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Stakeholder Engagement and Communication checklist



WHAT GOOD LOOKS LIKE



EARLY INCLUSION

Clinical staff and admin teams are included early in workflow design and feedback loops.



GOOD PATIENT INFORMATION

Patient information exists in inclusive formats (posters/FAQs) and clinicians can explain AVT confidently.



VISIBLE CHAMPIONS

Champions are visible, supported and used as peer-facing demonstrators.



EARLY ENGAGEMENT

Engagement starts early enough to shape decisions.



IDENTIFIED STAKEHOLDERS

All stakeholders are identified by impact, not just seniority.



RISKS IF YOU DON'T DO THIS



RUMOURS AND MISINTERPRETATION

Rumour and misinterpretation regarding increased workload and admin replacement, become narratives.



FRICTION CAUSES REWORK

Workflow issues surface late and create friction and rework.



INCREASED PATIENT CONFUSION

Patient confusion and uncertainty increases complaints or reduces consent.



MISSED RISKS

Critical risks or situations where AVT might struggle in practice (such as complex consultations or noisy environments) are missed.



MONITORING CEASE

Monitoring stops after AVT use begins rather than continuing in live use.



TOP TIPS

TOP TIP 1 Engage early

TOP TIP 2 Use clear role specific messaging

TOP TIP 3 Explain AVT in plain language



LINKS AND TOOLS

Stakeholder Engagement and Comms Resource

NHS Futures Change Management Toolkit Tool





Process Change

Design the workflow before you introduce the technology

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Design processes to enable AVT



PATHWAY 2 – We have piloted AVT and need to scale up

Standardise processes for consistent use



PATHWAY 3 – AVT is live, but adoption is patchy

Fix process issues affecting use



WHAT IT IS

Process Change supports the redesign of end-to-end workflows, so AVT fits into real practice and improves flow, quality and experience rather than adding extra steps.



USER INSIGHT

“ Process mapping and workflow design often happened too late, with limited input from those directly affected, leading to AVT being retrofitted into existing ways of working. ”



WHY THIS MATTERS



AVT benefit is realised through redesign of end-to-end workflows, not by inserting a tool into old habits.



When workflows don't align, it leads to duplication (parallel note-taking) and heavy editing, reducing confidence.



When people are involved in the design, they become advocates.



QUICK REALITY CHECK

Can staff explain the new process in simple terms?

Are workarounds visible and being addressed?

Are teams using AVT by default or as an optional extra?

Have people moved away from the old ways of working?





Process Change

What good looks like – and what's at risk if we don't get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Process Change and Ways of Working checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Process Change and Ways of Working checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Process Change and Ways of Working checklist



WHAT GOOD LOOKS LIKE



RECOGNISE GAPS

As-Is/To-Be workflows are mapped and variations between teams are recognised.



TAILOR TEMPLATES

Templates and outputs are deliberately tailored to local workflow requirements.



RETIRE OLD WORKAROUNDS

Old workarounds are identified and actively retired as the new process stabilises.



STAFF INVOLVEMENT

Involve the right staff in mapping workflows



CONTINUOUS IMPROVEMENT LOOP

Feedback from users and operations are used to refine the process.



RISKS IF YOU DON'T DO THIS



WORKAROUNDS BECOME NORMALISED

Workarounds become normalised and difficult to reverse.



INCONSISTENT PROCESSES

Safety and governance risks increase due to inconsistent process routes and identifiers.



LOST EFFICIENCY GAINS

Efficiency gains are lost due to parallel processes and rework.



POOR SCALABILITY

AVT may work in isolated cases but fail at broader rollout.



STAKEHOLDER RESISTANCE LATER

Adoption becomes harder once expectations and habits are fixed.



TOP TIPS

TOP TIP 1 Map reality (as is) with the right people

TOP TIP 2 Co-design and validate the future state

TOP TIP 3 Test with real users and real exceptions



LINKS AND TOOLS

Process Change Resource

NHS Futures Change Management Toolkit Tool





Change Impact

Be clear how AVT changes ways of working and workflows so staff can adapt with confidence

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Understand who is impacted and how



PATHWAY 2 – We have piloted AVT and need to scale up

Track change impact across teams



PATHWAY 3 – AVT is live, but adoption is patchy

Address impacts limiting adoption



WHAT IT IS

Change impact identifies who will be affected by AVT, how ways of working will change and what support is needed to adapt safely and confidently.



USER INSIGHT



AVT reshaped both culture and practice by redesigning workflows and reducing cognitive load, enabling clinicians to focus on patient care.



WHY THIS MATTERS



Identifying impact early builds credibility and reduces fear driven resistance.



Targeted support is possible only when the change impact is understood for each service/care setting.



Workforce concerns become adoption risks if not addressed transparently.



QUICK REALITY CHECK

Are you able to clearly state what the changes are for each role?

Have you tested the impact against actual day to day tasks?

Do you know whether the impact is minor, moderate or significant for each role?

Are managers able to explain the impact and answer practical questions?





Change Impact

What good looks like – and what’s at risk if we don’t get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Change Impact and Risk checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Change Impact and Risk checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Change Impact and Risk checklist



WHAT GOOD LOOKS LIKE



SUPPORT IDENTIFIED

Teams can describe what feels harder/easier and what supports exist during the “dip”.



PLANNED WORKFLOW REDESIGN

Admin workflow redesign is planned and communicated as role evolution rather than job loss.



IDENTIFIED ENVIRONMENTS

Environment and device requirements are identified and addressed before broader rollout.



CLEAR VIEW

There is a clear view of who and what is affected by the change.



ALIGNED TRAINING AND SUPPORT

Training, support and comms are aligned to the level of impact.



RISKS IF YOU DON'T DO THIS



RISING FEAR

Fear and disengagement build quietly, especially among admin teams.



UNMANAGEABLE EFFORTS

Early adopters disengage before benefits are seen if the early effort isn’t supported.



WORKFLOW FRICTION

Workflow friction is blamed on people instead of being treated as change impact to address.



MISSED STAKEHOLDERS

Key stakeholders are missed or engaged too late.



GO/NO GO DECISIONS AFFECTED

Impact results do not influence go/no go decisions.



TOP TIPS

TOP TIP 1 Provide clarity on what the changes will be

TOP TIP 2 Identify impacts early on

TOP TIP 3 Focus support on high impact areas



LINKS AND TOOLS

Change Impact Resource

NHS Futures Change Management Toolkit Tool





Risk Management

Manage risks early, especially around safety, accuracy and clinical responsibility

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Identify the risks before rollout begins



PATHWAY 2 – We have piloted AVT and need to scale up

Monitor and manage emerging risks



PATHWAY 3 – AVT is live, but adoption is patchy

Address risks impacting adoption



WHAT IT IS

Risk Management focuses on identifying and addressing people, workflow and adoption risks that could limit safe use, confidence or benefits realisation.



USER INSIGHT



Risk wasn't a barrier to adoption if it was actively managed through practical solutions and strong clinical oversight.



WHY THIS MATTERS



Trust and adoption collapse quickly after incidents if risks are not visible, owned and actively managed.



Risk management protects patients and staff by making accountability explicit and auditable.



Managing risk enables scale; it turns “pilot caution” into controlled expansion.



QUICK REALITY CHECK

Have practical adoption, people and delivery risks been identified?

Is someone accountable for monitoring and acting on each risk?

Are risks incorporated in regular project meetings?

Are risk discussions driving changes to plans, comms and training?





Risk Management

What good looks like – and what’s at risk if we don’t get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Change Impact and Risk checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Change Impact and Risk checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Change Impact and Risk checklist



WHAT GOOD LOOKS LIKE



EARLY PLANNED PROCESSES

DPIA, clinical safety processes, consent approach and hazard workshops are planned and executed early.



HUMAN IN THE LOOP

“Human in the Loop” review and approval expectations are consistently reinforced in training as standard clinical practice.



DOCUMENTED RISKS

Risks/mitigations are documented and revisited as adoption scales.



QUICK ESCALATIONS

Issues, incidents and exceptions are escalated quickly.



CONTINUAL MONITORING

Monitoring continues after launch.



RISKS IF YOU DON'T DO THIS



MISSED ERRORS

Medication/summary errors are missed or misinterpreted, creating safety incidents.



INCONSISTENCIES

Consent practice becomes inconsistent and fails audit expectations.



SPREADING WORKAROUNDS

Workarounds spread without governance oversight.



AFFECTED DECISIONS

Results do not influence go/no go or risk acceptance decisions.



UNUSED FINDINGS

Findings are not severity rated or prioritised.



TOP TIPS

TOP TIP 1 Give each risk a named owner

TOP TIP 2 Turn each mitigation into a practical activity

TOP TIP 3 Address risks before they affect rollout



LINKS AND TOOLS

Risk Management Resource

NHS Futures Change Management Toolkit Tool





Skills and Learning

Build confidence in practice not just through training

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Build skills and learning based on real use



PATHWAY 2 – We have piloted AVT and need to scale up

Reinforce skills and learning through feedback



PATHWAY 3 – AVT is live, but adoption is patchy

Support skills gaps affecting use



WHAT IT IS

Skills and Learning build confidence in using AVT safely and effectively by supporting new habits and ways of working, not just technical training.



USER INSIGHT

“ Training worked best when flexible and close to practice; group sessions were poorly attended in some contexts, with 1:1 and ‘at the elbow’ support being more effective. ”



WHY THIS MATTERS



Training attendance readiness; confidence and supported use predict adoption.



Skills must include workflow habits and template use, not just “how to click record.”



Building internal capability reduces dependency on suppliers and accelerates scale.



QUICK REALITY CHECK

Is training designed around real workflows, not just system features?

Can staff apply what they have learned in practice?

Are staff able to get help when they need it?

Are teams developing local capability?





Skills and Learning

What good looks like – and what’s at risk if we don’t get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Skills, Learning, Capability and Confidence checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Skills, Learning, Capability and Confidence checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Skills, Learning, Capability and Confidence checklist



WHAT GOOD LOOKS LIKE



ROLE BASED TRAINING

Training is role-based and reinforces “Human in the Loop” review and approval of AVT outputs, with clear clinical accountability.



AT THE ELBOW

“At the elbow” or in-service support exists for early use and those lacking confidence.



HELP

Refreshers and rapid help routes exist; small irritants are fixed quickly.



PREPARATION FOR NEW WAYS OF WORKING

Frontline teams are prepared for new ways of working.



PRE-GO LIVE ASSESSMENTS

Confidence and readiness are assessed before go live.



RISKS IF YOU DON’T DO THIS



NON-ADOPTION

People attend training but still don’t adopt because confidence is low and support isn’t available.



MIXED MESSAGES

Mixed messages on “Human in the Loop” review and approval of AVT outputs can lead to unsafe practice or avoidance.



SUPPLIER RELIANCE

Reliance on suppliers persists because local super-user capability is not built.



LATE IDENTIFICATION

Skills gaps are identified too late.



GENERIC TRAINING

Training is too generic and not tailored to real use.



TOP TIPS

TOP TIP 1 Tailor training to roles and specialties

TOP TIP 2 Use real tasks and scenarios

TOP TIP 3 Use flexible multi-format training approaches



LINKS AND TOOLS

NHS Futures Change Management Toolkit Tool





Change Adoption

Focus on how AVT is used in practice, not just whether it is used

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Define what good adoption looks like



PATHWAY 2 – We have piloted AVT and need to scale up

Monitor adoption across teams



PATHWAY 3 – AVT is live, but adoption is patchy

Address inconsistent adoption



WHAT IT IS

Change adoption is the extent to which AVT is used confidently and consistently in day-to-day practice beyond go-live. It involves regularly reviewing real usage and feedback to identify where support or reinforcement is needed.



USER INSIGHT



Adoption improved when organisations looked beyond usage data to understand user experience and variation in use, using structured pilots as learning opportunities with clear decision points rather than scaling up too early.



WHY THIS MATTERS



If people don't use AVT properly, you won't get the benefits and understanding why, helps you fix issues and scale safely.



Understanding “why not” enables targeted support for low confidence, workarounds and resistance.



Clear adoption decision points support safe scaling and protects trust.



QUICK REALITY CHECK

Is AVT being used as intended in real clinical workflows?

Is use consistent across different teams and roles?

Do staff feel confident using AVT in their day-to-day work?

Do you understand why some staff are not using AVT?





Change Adoption

What good looks like – and what's at risk if we don't get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Change Adoption checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Change Adoption checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Change Adoption checklist



WHAT GOOD LOOKS LIKE



ACTIVE MONITORING

Adoption is actively monitored using usage data and feedback from high and low users.



TARGETED RIENFORCEMENT

Targeted support is put in place where needed to support confident and consistent use.



LEARNING IN PRACTICE

Use early learning to refine support and strengthen stakeholder confidence.



VISIBLE SUPPORT

Champions are active post go-live providing targeted support to address issues and improve adoption.



CONSISTENT USE

AVT is used in a consistent way across teams and services in day-to-day practice.



RISKS IF YOU DON'T DO THIS



UNREALISED BENEFITS

If adoption is inconsistent, the benefits of AVT will not be realised.



PATCHY USE

Variation in how AVT is used across teams leads to inconsistent use, reducing confidence.



HIDDEN ISSUES

Without understanding why people are not using AVT, underlying issues are missed.



PERSISTENT WORKAROUNDS

Low confidence, workarounds and resistance persist without targeted reinforcement, undermining effective use.



SCALING TOO EARLY

Without clear adoption insights and decision points, organisations risk scaling too quickly.



TOP TIPS

TOP TIP 1 Be specific about what people need to do

TOP TIP 2 Equip managers to reinforce messages

TOP TIP 3 Share good examples of change from live teams



LINKS AND TOOLS

Change Adoption Resource

NHS Futures Change Management Toolkit Tool





Transfer to Business as Usual (BAU)

Plan how AVT will be owned and sustained after Go-Live

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Plan long-term ownership and support model



PATHWAY 2 – We have piloted AVT and need to scale up

Embed AVT into standard processes



PATHWAY 3 – AVT is live, but adoption is patchy

Re-establish AVT as the new normal



WHAT IT IS

Transfer to BAU ensures AVT moves from a time-limited rollout into routine clinical and operational practice, with clear ownership, ongoing support and sustained adoption.



USER INSIGHT

“Where AVT remained owned only by a project or digital team, workflow and behaviour issues resurfaced without clear operational accountability.”



WHY THIS MATTERS



Without a clear BAU transition, AVT risks becoming “something we tried” rather than how we work.



BAU ownership ensures clinical safety behaviours are maintained under everyday operational pressure.



Embedding AVT into BAU enables continuous optimisation (templates, workflows, support) rather than freezing practice at go-live.



QUICK REALITY CHECK

Can you name who owns AVT in BAU?

Is AVT being used consistently across teams?

Do staff know where to go when something goes wrong?

Are issues being fixed and improvements made?





Transfer to Business As Usual (BAU)

What good looks like – and what's at risk if we don't get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Transfer to BAU checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Transfer to BAU checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Transfer to BAU checklist



WHAT GOOD LOOKS LIKE



CLEAR OWNERSHIP

There is named BAU ownership for adoption, safety, support and optimisation.



ACTIVE MONITORING

Adoption, confidence and safety signals are reviewed.



CONTINUOUS IMPROVEMENT

Feedback loops and reinforcement activity continue after go-live, with visible fixes and updates.



VISIBLE VALUE

AVT performance and benefits are discussed within routine service or operational forums.



ACCESSIBLE SUPPORT

People know where to go for support and how to get help quickly



RISKS IF YOU DON'T DO THIS



UNCLEAR OWNERSHIP

Adoption slowly erodes as early enthusiasm fades and no one "owns" sustained use.



REDUCES OVERSIGHT

Safety practices (review and approval of AVT outputs and consent recording) weaken under pressure without reinforcement.



PERSISTENT ISSUES

Workflow and template issues persist, leading to frustration and workarounds.



UNPROVEN VALUE

Organisations struggle to evidence value, making further scale or funding decisions harder.



LIMITED SUPPORT

Support routes are unclear, so issues persist or are ignored.



TOP TIPS

TOP TIP 1 Make ownership explicit

TOP TIP 2 Embed AVT into everyday work

TOP TIP 3 Continue monitoring after the project closes



LINKS AND TOOLS

Transfer to BAU Resource

NHS Futures Change Management Toolkit





Change Network

Use trusted peers as champions to drive adoption and improvement

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Build local champions and support network



PATHWAY 2 – We have piloted AVT and need to scale up

Use network to share feedback and learning



PATHWAY 3 – AVT is live, but adoption is patchy

Reactivate network to support teams



WHAT IT IS

A Change Network brings together trusted clinical, operational and admin champions to share clear messages, surface issues early and support peer to peer adoption of AVT.



USER INSIGHT

“Adoption spread through trusted peer to peer influence with champions often emerging organically who helped to drive uptake and sustain engagement over time.”



WHY THIS MATTERS



Building trust enabling two-way engagement helps to drive adoption by involving staff in the change from the start.



Local networks help to build local confidence and support by using trusted peers.



Champions can help to surface workflow and template issues early through frontline insight, speeding up improvements.



QUICK REALITY CHECK

Do you have champions across the right roles, teams and locations?

Do network members know what they have been asked to do?

Has protected time been discussed and agreed with managers?

Have feedback channels been set up and shared?





Change Network

What good looks like – and what's at risk if we don't get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Change Network checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Change Network checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Change Network checklist



WHAT GOOD LOOKS LIKE



VISIBLE CHAMPIONS

Champions are visible and active locally with peer learning, coaching and demonstration driving confidence and adoption as part of a Change Network.



TWO WAY ENGAGEMENT

Two-way engagement is working with clear, consistent messages and early feedback shaping decisions.



USE OF INSIGHTS

Insights are used to improve rollout, with champions surfacing issues early and informing workflow and template refinement.



SUPPORTS ADOPTION

The network supports adoption before, during and after launch.



HELP WITH VALIDATION

Champions help validate real world scenarios and user impacts.



RISKS IF YOU DON'T DO THIS



LIMITED SUPPORT

Limited local support and influence. Change stays centralised with no trusted peers to explain, support and encourage adoption.



LOW TRUST

Low trust and engagement. People don't feel involved or heard so confidence drops and resistance increases.



NO PEER SUPPORT

Poor adoption and inconsistent use. Without peer support and reinforcement, take up is patchy and new ways of working don't stick.



LACK OF REPRESENTATION

Champions are not representative of affected teams.



STOPPED ACTIVITY

Network activity stops after launch.



TOP TIPS

TOP TIP 1 Create regular touchpoints

TOP TIP 2 Recognise and celebrate champions

TOP TIP 3 Use the network to surface blockers



LINKS AND TOOLS

NHS Futures Change Management Toolkit Tool





WHAT'S IN IT FOR ME (WIIFM) BENEFITS – page 1 of 2

WIIFM Benefits

Make the benefits real, relevant and meaningful for everyone

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Show how AVT helps people's roles



PATHWAY 2 – We have piloted AVT and need to scale up

Reinforce personal benefits from pilot experiences



PATHWAY 3 – AVT is live, but adoption is patchy

Rebuild belief in the WIIFM benefits



WHAT IT IS

What's in it for me (WIIFM) translates AVT benefits into clear, personal messages so staff can see how the change will make their own work easier or better.



USER INSIGHT

“WIIFM benefits landed more effectively when they reflected real practical clinical experience – reduced cognitive load, increased patient focus and clinical safety.”



WHY THIS MATTERS



WIIFM messaging is a primary driver of engagement and can support resistance reduction.



Role-specific benefits help reduce fear about workforce impact and job loss narratives.



Credible WIIFM support safer adoption by reinforcing correct use and “Human in the Loop” review and approval of AVT outputs.



QUICK REALITY CHECK

Do people understand what's in it for them?

Are the benefits relevant to the real workforce pressures?

Are we communicating WIIFM benefits in the right way?

Are people showing they understand the reason for the AVT introduction





WHAT'S IN IT FOR ME (WIIFM) BENEFITS – page 2 of 2

WIIFM Benefits

What good looks like – and what's at risk if we don't get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Benefits and WIIFM checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Benefits and WIIFM checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Benefits and WIIFM checklist



WHAT GOOD LOOKS LIKE



STAFF EXPLANATION

Staff can explain the benefits in their own words, not just repeat generic transformation language.



REAL DAY TO DAY IMPROVEMENTS

Communications focus on real day to day improvements.



CLEAR ADMIN IMPACTS

Admin impacts are clearly explained, including how roles may evolve and how staff will be supported.



FEEDBACK LED ADOPTION

Concerns are captured and used to refine messaging, training and support.



ROLE SPECIFIC BENEFITS

Benefits are tailored by audience, especially for clinicians.



RISKS IF YOU DON'T DO THIS



DOMINATING MYTHS

Productivity enforcement myths dominate and reduce uptake.



INCREASED ANXIETY

Admin staff assume AVT is workforce reduction leading to increased anxiety.



ENTHUSIASM FADES

Benefits remain theoretical; enthusiasm fades quickly.



TRAINING LANDS POORLY

Staff attend training but do not apply AVT because the 'Why' is unclear.



MOMENTUM FADES

Early interest drops off before AVT becomes embedded in ways of working.



TOP TIPS

TOP TIP 1 Share early wins and peer stories

TOP TIP 2 Connect benefits to real situations

TOP TIP 3 Tailor WIIFM by role



LINKS AND TOOLS

WIIFM Benefits Resource

NHS Futures Change Management Toolkit





Ways of Working

Make AVT part of everyday practice – set clear expectations and co-design locally

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Define how roles and work will change



PATHWAY 2 – We have piloted AVT and need to scale up

Embed new ways into daily practice



PATHWAY 3 – AVT is live, but adoption is patchy

Understand and then reset new ways of working



WHAT IT IS

Ways of Working define the new behaviours, habits and expectations needed to use AVT safely and consistently across different clinical settings.



USER INSIGHT



AVT only transformed practice when teams changed habits — otherwise old ways persisted.



WHY THIS MATTERS



Inconsistent practice can emerge across teams and settings.



Clinicians slip back into old habits rewriting notes or just dictation.



Safety risks increase when “Human in the Loop” review and approval and clinical ownership drop when it gets busy.



QUICK REALITY CHECK

Can staff explain how AVT fits with the workflow?

Do supporting materials match actual practice?

Are workarounds being spotted?

Is feedback being used to refine the workflow?





Ways of Working

What good looks like – and what’s at risk if we don’t get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Process Change and Ways of Working checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Process Change and Ways of Working checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Process Change and Ways of Working checklist



WHAT GOOD LOOKS LIKE



EXPLICIT AGREEMENTS

Teams explicitly agree how AVT is used (verbalising what isn’t said aloud, how it is checked and any exceptions).



COACHING

New habits are actively supported through coaching, not just left after training.



CONTINUAL IMPROVEMENTS

Workflows and templates keep improving based on real use and feedback.



ROLE CLARITY

Staff understand who does what, what changes and what stays the same.



UPDATED STANDARD OPERATING PROCEDURES (SOPS)

SOPs are updated to incorporate the new ways of working.



RISKS IF YOU DON’T DO THIS



PRACTICE VARIATIONS

Variation in practice across teams and settings.



OLD HABITS RETURN

Reversion to old habits (e.g. double note-taking, dictation only).



SAFETY RISK

Increased safety risk when checks and clinical ownership drop under pressure.



ROLE AMBIGUITY

Teams lack clarity on responsibilities and decision making.



CHANGE FATIGUE

Staff see AVT as extra work rather than a better way of working.



TOP TIPS

TOP TIP 1 Identify manual steps

TOP TIP 2 Use team meetings to embed the new habits

TOP TIP 3 Co-design local ways of working



LINKS AND TOOLS

Ways of Working Resource

NHS Futures Change Management Toolkit Tool





READINESS AND PLANNING – page 1 of 2

Readiness and Planning

Assess readiness early and plan for adoption not just implementation

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Assess readiness before starting pilot



PATHWAY 2 – We have piloted AVT and need to scale up

Plan rollout based on area readiness



PATHWAY 3 – AVT is live, but adoption is patchy

Revisit poor rollout decisions



WHAT IT IS

Readiness and Planning help teams assess whether services are truly ready for AVT and identify actions to reduce disruption and build confidence before go-live.



USER INSIGHT

“Readiness worked best when clear readiness criteria enabled conditional go-live and better adoption.”



WHY THIS MATTERS



Readiness protects first user experience and confidence, reducing early drop off.



It surfaces environment and workflow issues early (noise, devices, templates) before they become adoption killers.



It creates shared accountability across clinical/admin/Ops rather than relying on enthusiasts.



It enables scale as a repeatable process with consistent go/no go discipline.



QUICK REALITY CHECK

Is readiness understood?

Have any gaps been identified?

Can managers explain AVT and reinforce adoption?

Are feedback and usage signals being monitored to adjust the plan?



The [Playbook](#) defines the readiness criteria that should be assessed. This section provides guidance on how to assess, evidence and improve readiness.





READINESS AND PLANNING – page 2 of 2

Readiness and Planning

What good looks like – and what’s
at risk if we don’t get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are
starting AVT for the
first time

Readiness and
Planning checklist



PATHWAY 2 – We
have piloted AVT and
need to scale up

Readiness and
Planning checklist



PATHWAY 3 – AVT is
live, but adoption is
patchy

Readiness and
Planning checklist



WHAT GOOD LOOKS LIKE



READINESS EVIDENCE

Go-live is conditional on readiness evidence, not date-driven.



MITIGATION PLANNING

Connectivity, device and environment risks are assessed and mitigated before scale.



EXPLICIT OWNERSHIP

Ownership is explicit (who supports users, who fixes issues, how escalation works).



SUPPORT IS READY

FAQs, floorwalking, help channels and escalation routes are in place from day one.



PROGRESS IS TRACKED

Adoption, confidence and usage indicators are monitored so plans can be adjusted quickly.



RISKS IF YOU DON'T DO THIS



DAMAGED CONFIDENCE

Early failures damage confidence and create unnecessary resistance.



ISSUES WITH INFRASTRUCTURE

Infrastructure issues are misread as “people don’t like AVT”.



PATCHY ADOPTION

Adoption becomes patchy with pockets of use and non-use.



BENEFITS DELAYED

Benefits are hard to evidence if baselines and measurement plans were not agreed early.



SUPPORT OVERLOAD

Help channels become overwhelmed because common questions and issues are not anticipated.



TOP TIPS

TOP TIP 1 Make readiness visible and measurable

TOP TIP 2 Plan Go-Live on readiness criteria

TOP TIP 3 Use readiness to drive adoption



LINKS AND TOOLS

**Readiness and
Planning Resource**

**NHS Futures Change
Management Toolkit
Tool**





Feedback Cycles

Act on feedback quickly and visibly so small issues don't become adoption blockers

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Capture feedback early and often



PATHWAY 2 – We have piloted AVT and need to scale up

Use feedback to improve and adapt



PATHWAY 3 – AVT is live, but adoption is patchy

Rebuild feedback loops to support adoption



WHAT IT IS

Feedback creates structured ways for staff to share what is working and what is not, so learning can be acted on quickly to improve adoption and experience.



USER INSIGHT



Iterative approaches, regular supplier engagement and workflow adjustments based on user feedback helped to improve adoption.



WHY THIS MATTERS



Feedback cycles are how adoption issues are detected early and turned into improvements before resistance builds.



Feedback supports safe practice by surfacing any issues such as consent and “Human in the Loop” review and approval in day-to-day use.



Feedback helps refine templates and workflows to match how teams actually work, enabling optimisation.



QUICK REALITY CHECK

Is the AVT feedback route clear to staff?

Is feedback reviewed on a regular basis?

Are adoption barriers being identified before they become embedded?

Are workflow changes tested with users before wider rollout?





Feedback Cycles

What good looks like – and what’s at risk if we don’t get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Stakeholder Engagement and Communication checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Stakeholder Engagement and Communication checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Stakeholder Engagement and Communication checklist



WHAT GOOD LOOKS LIKE



EXISTENCE OF FEEDBACK ROUTES

Feedback routes exist beyond surveys (champions, huddles, office hours) and are used regularly.



VISIBLE FIXES

Feedback results in visible fixes (workflow, templates, training refreshers).



ALL USERS INCLUDED

Feedback includes high and low users so “why not” is understood.



CLEAR OWNERSHIP

Named people are responsible for collating, reviewing and acting on AVT feedback.



SUPPLIER ENGAGEMENT

Suppliers are involved early where technical or product issues affect adoption.



RISKS IF YOU DON'T DO THIS



ADOPTION KILLERS

Small irritants become adoption killers because there's no rapid fix route.



CULTURAL RESISTANCE

Adoption drops without warning and is mislabelled as cultural resistance.



SAFETY ISSUES UNNOTICED

Safety and consent compliance issues persist unnoticed until audit/incident.



WORKAROUNDS BECOME NORMAL

Staff develop unofficial fixes instead of feeding issues into a structured improvement cycle.



LOW TRUST

Staff stop raising issues because they do not believe feedback will lead to action.



TOP TIPS

TOP TIP 1 Make feedback easy and routine for staff

TOP TIP 2 Act on feedback quickly to build trust

TOP TIP 3 Use feedback to improve adoption and experience



LINKS AND TOOLS

Feedback Cycles Resource

NHS Futures Change Management Toolkit Tool





Resistance Management

Normalise resistance – listen,
act early and build trust
through visible change

WHERE THIS MATTERS

PATHWAY 1 - We are
starting AVT for the
first time

Identify concerns
as early as
possible



PATHWAY 2 – We
have piloted AVT and
need to scale up

Address
resistance as it
emerges



PATHWAY 3 – AVT is
live, but adoption is
patchy

Resolve resistance
issues affecting
adoption



WHAT IT IS

Resistance Management
recognises resistance as a
normal response to change
and provides practical ways
to understand and address
concerns early.



USER INSIGHT



Poor messaging
increased resistance
especially when AVT
was linked to additional
workload or productivity.



WHY THIS MATTERS



Resistance occurs where confidence,
workflows or messaging need attention.
If ignored it could lead to passive dis-
engagement.



Addressing resistance early prevents
workarounds and unsafe shortcuts.



Honest conversations preserve goodwill
and enable scale decisions to be
evidence led.



QUICK REALITY CHECK

Are concerns being heard
early?

Is the route cause of
resistance understood?

Are WIIFM benefits being
made locally relevant?

Is resistance being managed
proactively?





Resistance Management

What good looks like – and what’s
at risk if we don’t get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are
starting AVT for the
first time

Resistance
Management
checklist



PATHWAY 2 – We
have piloted AVT and
need to scale up

Resistance
Management
checklist



PATHWAY 3 – AVT is
live, but adoption is
patchy

Resistance
Management
checklist



WHAT GOOD LOOKS LIKE



EXPLICIT MESSAGING

Messaging is explicit about what AVT will and won’t do, including that it
doesn’t automatically mean more patients or more appointments.



PRACTICAL SUPPORT

Resistant users receive practical support and demonstration, not blame.



LEADERS REINFORCE OWNERSHIP

Governance and leaders reinforce that behavioural and workflow
ownership sits with services, not just digital teams.



PEER ADVOCACY

Early adopters and trusted colleagues help demonstrate practical benefits
in real settings.



TRUST BUILDS OVER TIME

Staff see that their concerns lead to meaningful action, making AVT feel
safer and more credible.



RISKS IF YOU DON’T DO THIS



POLITICAL OPPOSITION

Resistance builds into political opposition and low uptake.



INFRASTRUCTURE ISSUES

Infrastructure and workflow issues are wrongly treated as “people
problems”.



UNSAFE SHORTCUTS

When under pressure, proper processes are avoided and risk increases



RESISTANCE IS IGNORED

Concerns are dismissed as negativity rather than treated as useful
intelligence.



BENEFITS FEEL ABSTRACT

Staff hear broad messages about AVT but do not see practical value in
their own role.



TOP TIPS

TOP TIP 1 Treat resistance as insights

TOP TIP 2 Address practical barriers

TOP TIP 3 Utilise the trusted Change Champions



LINKS AND TOOLS

Resistance Management Resource

NHS Futures Change Management Toolkit Tool





Reinforcement

Close the loop – act on feedback to sustain adoption

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Understand trends of early use and behaviours



PATHWAY 2 – We have piloted AVT and need to scale up

Strengthen consistent use in practice



PATHWAY 3 – AVT is live, but adoption is patchy

Reinforce the why and how for AVT



WHAT IT IS

Reinforcement sustains AVT adoption by acting on feedback, building confidence, fixing issues early and celebrating success.



USER INSIGHT



Strong feedback loops and pausing to learn helped to resolve issues early and supported more effective scaling.



WHY THIS MATTERS



Reinforcement sustains behaviour change and prevents “tried once and stopped” patterns.



Turns adoption insights into action i.e. fixes, support and improvements.



Recognising success builds confidence and encourages wider AVT use.



QUICK REALITY CHECK

Is AVT use reviewed and acted on?

Are issues fixed quickly from feedback?

Is additional support targeted where uptake is low?

Are successes being shared to drive adoption?





Reinforcement

What good looks like – and what's at risk if we don't get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are starting AVT for the first time

Reinforcement checklist



PATHWAY 2 – We have piloted AVT and need to scale up

Reinforcement checklist



PATHWAY 3 – AVT is live, but adoption is patchy

Reinforcement checklist



WHAT GOOD LOOKS LIKE



TARGETED REINFORCEMENT ACTIONS

Regular review of adoption plus feedback is used to target reinforcement actions.



FIXED PAIN POINTS

Known pain points are fixed quickly and communicated back to users.



ONGOING BENEFITS

Ongoing optimisation and benefits tracking continues beyond initial rollout. Benefits and successes are shared widely to reinforce value.



AVT BECOMES ROUTINE

AVT is embedded into everyday workflows, meetings, supervision and operational conversations.



DATA GUIDES REINFORCEMENT

Usage data, feedback and adoption patterns show where extra reinforcement is needed.



RISKS IF YOU DON'T DO THIS



FALLEN ADOPTION

Adoption falls after initial friction because no one closes the loop.



ENTRENCHED BEHAVIOURS

Small irritants become entrenched behaviours and workarounds.



UNPROVEN BENEFITS

Benefits remain unproven and scaling loses momentum.



OLD HABITS RETURN

Teams revert to previous ways of working when AVT is not embedded into daily routines.



ONE OFF TRAINING

Staff receive go live training but no refresher support as context changes.



TOP TIPS

TOP TIP 1 Turn feedback into action fast

TOP TIP 2 Target support where it is needed the most

TOP TIP 3 Share AVT wins and celebrate success



LINKS AND TOOLS

Reinforcement Resource

NHS Futures Change Management Toolkit Tool





CAPABILITY AND CONFIDENCE –
page 1 of 2

Capability And Confidence

Confidence grows when AVT makes work easier in practice

WHERE THIS MATTERS

PATHWAY 1 - We are starting AVT for the first time

Build confidence in using AVT



PATHWAY 2 – We have piloted AVT and need to scale up

Strengthen confidence through usage



PATHWAY 3 – AVT is live, but adoption is patchy

Restore confidence where usage has dropped



WHAT IT IS

Capability and Confidence focus on building the organisational skills and confidence needed to make AVT a trusted routine part of everyday practice.



USER INSIGHT



Adoption was driven by proven performance in practice through proof of concepts, whilst reliability issues quickly reduced trust.



WHY THIS MATTERS



Capability and confidence determines whether AVT becomes routine practice or use remains limited.



Confidence supports safe “Human in the Loop” review and approval of AVT outputs, even under pressure.



Internal capability reduces supplier dependence and helps scale up AVT.



QUICK REALITY CHECK

Can staff use AVT without step-by-step instructions?

Do confidence levels match training completion?

Have staff had safe practice time?

Is peer support available locally?



Champions play a critical role in building local capability and confidence through peer-to-peer support.





CAPABILITY AND CONFIDENCE – page 2 of 2

Capability And Confidence

What good looks like – and what’s
at risk if we don’t get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are
starting AVT for the
first time

Skills, Learning,
Capability and
Confidence
checklist



PATHWAY 2 – We
have piloted AVT and
need to scale up

Skills, Learning,
Capability and
Confidence
checklist



PATHWAY 3 – AVT is
live, but adoption is
patchy

Skills, Learning,
Capability and
Confidence
checklist



WHAT GOOD LOOKS LIKE



RELIABLE AVT USE

Users can use AVT reliably in real settings and maintain full clinical responsibility through “Human in the Loop” review and approval.



CHANGE CHAMPIONS SUPPORT

Local champions/super-users exist and are supported with customisation skills.



IMPROVED COMPLIANCE BEHAVIOURS

Compliance behaviours (e.g. consent recording) improve through reinforcement and clearer workflows.



CONFIDENCE BUILDS GRADUALLY

Users are supported to move from basic familiarity to confident, routine use.



LEARNING IS REVISITED

Training is revisited as systems, workflows or staff needs change.



RISKS IF YOU DON'T DO THIS



STALLED ADOPTION

Early trust issues spread socially and adoption stalls.



UNSAFE PRACTICES

Unsafe practice arises when “Human in the Loop” review and approval and clinical accountability are not embedded.



VENDOR DEPENDENCY

Organisations remain dependent on vendors for day-to-day optimisation.



GENERIC TRAINING

Support does not reflect different roles, workflows or confidence levels.



LOW CONFIDENCE

Some users avoid AVT because they are uncomfortable with the technology.



TOP TIPS

TOP TIP 1 Make help easy to find

TOP TIP 2 Track confidence alongside usage

TOP TIP 3 Check staff are ready to use AVT



LINKS AND TOOLS

NHS Futures Change
Management Toolkit
Tool





Organisational Change Management Maturity

AVT success depends on managing
adoption, not just implementation.

WHERE THIS MATTERS

PATHWAY 1 - We are
starting AVT for the
first time

Assess Change
Maturity across
the organisation.



PATHWAY 2 – We
have piloted AVT and
need to scale up

Strengthen Change
capability to
support scaling up



PATHWAY 3 – AVT is
live, but adoption is
patchy

Use Change
principles to
support adoption



WHAT IT IS

Organisational Change
Management Maturity helps
organisations reflect on their
wider ability to manage and
sustain change, so change
adoption strengthens
long-term capability rather
than adding to change
fatigue.



USER INSIGHT

“ Breaking readiness into
clear repeatable categories
(leadership, governance,
technology, people and
service readiness)
supported more consistent
AVT success.



WHY THIS MATTERS



Change maturity informs how much
support and structure is needed for safe,
sustained change.



It prevents labelling organisations as
being resistant, when issues are related
to capability, resourcing or readiness
gaps.



It enables repeatable scaling by applying
consistent readiness criteria and
governance discipline.



QUICK REALITY CHECK

Is AVT being managed as a
people and workflow
change?

Is there enough change
capability?

Have stakeholders shaped
the approach?

Are managers and
champions equipped?





Organisational Change Management Maturity

What good looks like – and what's
at risk if we don't get it right

ARE YOU ON TRACK?

PATHWAY 1 - We are
starting AVT for the
first time

Organisational
Change
Management
Maturity Checklist



PATHWAY 2 – We
have piloted AVT and
need to scale up

Organisational
Change
Management
Maturity Checklist



PATHWAY 3 – AVT is
live, but adoption is
patchy

Organisational
Change
Management
Maturity Checklist



WHAT GOOD LOOKS LIKE



STRUCTURED READINESS

Structured readiness criteria are used before scaling.



USEFUL LEARNING CAPTURED

Learning is captured and reused through evidence and examples.



REPEATABLE CHANGE PRACTICES

Change practices become repeatable, with clear ownership, cadence and measures.



LEADERSHIP ALIGNMENT

Senior leaders are aligned on why AVT matters and what success look like.



BENEFITS ARE OWNED

Expected benefits are clearly defined, tracked and owned beyond go live.



RISKS IF YOU DON'T DO THIS



INCONSISTENT CHANGE

Change becomes reactive and inconsistent; fatigue increases as lessons are not implemented.



EVIDENCE LED DECISIONS

Scaling decisions become enthusiasm driven rather than evidence led.



GOVERNANCE GAPS

Governance and readiness gaps create avoidable disruption and loss of trust.



ADOPTION NOT ACTIVELY MANAGED

Focus stays on go live, with limited tracking of usage, confidence or barriers.



INSUFFICIENT CHANGE CAPABILITY

Teams lack the time, resource or skills to support adoption properly.



TOP TIPS

TOP TIP 1 Treat AVT as a people and workflow change

TOP TIP 2 Build local change capacity

TOP TIP 3 Align leaders and engage stakeholders



LINKS AND TOOLS

NHS Futures Change
Management Toolkit
Tool





CELEBRATING SUCCESS –
page 1 of 2

Celebrating Success

Seeing successes makes
adoption feel real

WHERE THIS MATTERS

PATHWAY 1 - We are
starting AVT for the
first time

Recognise early
wins and positive
outcomes



PATHWAY 2 – We
have piloted AVT and
need to scale up

Share success
stories to build
momentum



PATHWAY 3 – AVT is
live, but adoption is
patchy

Reinforce
success stories
to rebuild belief



WHAT IT IS

Celebrating successes not
only increases morale but it
builds support among those
that may need to "see it to
believe it".

Recognition is needed in
order to cement or reinforce
the change.



USER INSIGHT



Celebrating success was
most effective when staff
saw what worked from
their peers, as real
examples from colleagues
strengthened engagement
and adoption.



WHY THIS MATTERS



Saying exactly what went well helps
everyone feel proud and ready for the
next step.



It creates a culture of appreciation where
people feel valued, recognised and
encouraged to keep improving.



It reinforces positive behaviours by
showing what good looks like and
encouraging others to follow it.



QUICK REALITY CHECK

Are you celebrating genuine
progress, not just activity?

Are you recognising small
and large successes?

Do you have a criteria for
what success looks like?

Are you learning from
successes and not just
measuring them?



Success stories should reinforce adoption behaviours, benefits realised and improvements delivered through AVT.





Celebrating Success

What good looks like – and what’s
at risk if we don’t get it right

ARE WE ON TRACK?

PATHWAY 1 - We are
starting AVT for the
first time

Reinforcement
checklist



PATHWAY 2 – We
have piloted AVT and
need to scale up

Reinforcement
checklist



PATHWAY 3 – AVT is
live, but adoption is
patchy

Reinforcement
checklist



WHAT GOOD LOOKS LIKE



PROGRESS IS MADE VISIBLE

Achievements are shared clearly so people can see what has improved and why it matters.



USEFUL LEARNING CAPTURED

Celebrating success includes reflecting on what worked well, so the learning can be used again.



PEOPLE RECOGNITION

Recognise the people behind the success, including quieter contributions and team effort.



SUCCESS MEASURES DEFINED

Clear definitions for measuring success is shared and understood.



TIMELY RECOGNITION

Success is acknowledged soon after it happens, so staff feel their contribution has been noticed and valued.



RISKS IF YOU DON'T DO THIS



MOTIVATION DROPS

People may feel their effort goes unnoticed, which can reduce energy, commitment and enthusiasm.



PROGRESS BECOMES INVISIBLE

Without celebration, improvements and achievements can be overlooked or forgotten.



LEARNING IS LOST

If AVT does not pause to recognise success, it may also miss the opportunity to understand what worked well.



CONFIDENCE IS REDUCED

Staff may be less likely to take initiative or try new approaches if success is not acknowledged.



TEAM SPIRIT WEAKENS

A lack of recognition can reduce shared pride, connection and collaboration across AVT.



TOP TIPS

TOP TIP 1 Link Success to AVT values and standards

TOP TIP 2 Use Success to build momentum

TOP TIP 3 Keep recognition honest, sincere and personal



LINKS AND TOOLS

NHS Futures Change
Management Toolkit
Tool





ARE YOU ON TRACK? – VISION

Pathway 1 - We are starting AVT for the first time

Vision

Descriptor: Organisations are defining what AVT is for and why it matters for care and staff experience.

Are you on track?

- ☐ Define a clear purpose for AVT focused on improving patient care and clinician experience.
- ☐ Articulate how AVT improves interaction, workflow and time back to care.
- ☐ Align the vision with organisational priorities and local pressures.
- ☐ Position AVT as service transformation, not just a technology deployment.
- ☐ Develop a simple, repeatable explanation of AVT in plain English.
- ☐ Ensure leaders and early adopters communicate the same core message.
- ☐ Test understanding with frontline staff and refine the message based on feedback.
- ☐ Explain what will change in practice, not just why AVT exists for different staff groups.





ARE YOU ON TRACK? – VISION

Pathway 2 - We have piloted AVT and need to scale up Vision

Descriptor: Organisations are ensuring the vision is understood, consistent and reinforced across teams.

Are you on track?

- ☐ Ensure the vision is understood consistently across teams and services.
- ☐ Reinforce the purpose of AVT through leaders, champions and day-to-day conversations.
- ☐ Use real examples from early adopters to show how AVT improves care and experience.
- ☐ Challenge misconceptions such as 'just dictation' or 'just a productivity tool'.
- ☐ Keep the vision focused on patient outcomes and staff experience.
- ☐ Ensure teams understand how AVT changes behaviours and workflows.
- ☐ Repeat and refresh messaging regularly as rollout expands.



ARE YOU ON TRACK? – VISION

Pathway 3 - AVT is live but adoption is patchy Vision

Descriptor: Organisations are correcting misunderstanding and reinforcing the original purpose of AVT.

Are you on track?

- ☐ Ask staff what they believe AVT is for and identify inconsistencies.
- ☐ Identify where AVT is being used as dictation rather than as intended.
- ☐ Pinpoint where the vision has become diluted or misunderstood.
- ☐ Reset the purpose of AVT using simple, practical language.
- ☐ Reinforce the vision through visible local clinical leadership.



ARE YOU ON TRACK? – CASE FOR CHANGE

Pathway 1 - We are starting AVT for the first time Case for Change

Descriptor: Organisations are building a credible, evidence-based reason for AVT.

Are you on track?

- ☐ Define the core problem AVT is solving for your organisation.
- ☐ Use local evidence to show the scale of the problem.
- ☐ Link AVT to real frontline pain points for clinical and admin teams.
- ☐ Align the case for change with organisational priorities.
- ☐ Focus the case on staff experience and patient care, not just efficiency.
- ☐ Avoid over-promising productivity gains at this stage.
- ☐ Test whether frontline staff recognise and agree with the case being made.



ARE YOU ON TRACK? – CASE FOR CHANGE

Pathway 2 - We have piloted AVT and need to scale up Case for Change

Descriptor: Organisations are strengthening and validating the case using real evidence.

Are you on track?

- ☐ Update the case for change using real pilot data and staff experience.
- ☐ Show how AVT is improving workflow, experience or care in early adopters.
- ☐ Ensure the case is understood consistently across services.
- ☐ Tailor the case to different teams and specialties.
- ☐ Balance qualitative benefits with measurable indicators.
- ☐ Reinforce the case through leaders and champions.



ARE YOU ON TRACK? – CASE FOR CHANGE

Pathway 3 - AVT is live but adoption is patchy Case for Change

Descriptor: Organisations are reconnecting AVT to real pressures and needs.

Are you on track?

- ☐ Ask staff whether AVT is solving a meaningful problem for them.
- ☐ Identify where the original case for change no longer resonates.
- ☐ Pinpoint whether resistance is driven by experience, workflow or perception.
- ☐ Reset the case using local, credible examples.
- ☐ Challenge the perception that AVT adds work without value.



ARE YOU ON TRACK? – SENIOR SPONSORSHIP AND GOVERNANCE

Pathway 1 - We are starting AVT for the first time Senior Sponsorship and Governance

Descriptor: Organisations are establishing governance structures and decision-making for AVT.

Are you on track?

- ☐ Identify a credible clinical sponsor and an executive sponsor for AVT.
- ☐ Define clear ownership across clinical, operational, digital and governance teams.
- ☐ Establish a governance structure with the right representation.
- ☐ Agree how key decisions will be made and escalated.
- ☐ Involve Information Governance and clinical safety early.
- ☐ Align AVT governance to existing trust governance structures.
- ☐ Clarify expectations for clinician accountability and sign-off.
- ☐ Ensure sponsors can clearly explain why AVT matters.





ARE YOU ON TRACK? – SENIOR SPONSORSHIP AND GOVERNANCE

Pathway 2 - We have piloted AVT and need to scale up Senior Sponsorship and Governance

Descriptor: Organisations are maintaining active sponsorship and consistent governance as rollout expands.

Are you on track?

- ☐ Keep sponsors visible and active beyond initial rollout.
- ☐ Use governance forums to review adoption, risk and emerging issues.
- ☐ Ensure decision-making is timely and responsive to frontline issues.
- ☐ Maintain active involvement from clinical safety, IG and operational leaders.
- ☐ Align governance decisions across services to reduce fragmentation.
- ☐ Monitor where ownership is unclear or inconsistent.
- ☐ Support local leaders to take service-level ownership of adoption.



ARE YOU ON TRACK? – SENIOR SPONSORSHIP AND GOVERNANCE

Pathway 3 - AVT is live but adoption is patchy Senior Sponsorship and Governance

Descriptor: Organisations are fixing gaps in accountability, ownership and decision-making.

Are you on track?

- ☐ Identify where ownership is unclear or absent.
- ☐ Confirm who is responsible for addressing adoption issues locally.
- ☐ Re-engage sponsors where visibility or support has reduced.
- ☐ Use governance to prioritise and unblock barriers to use.
- ☐ Clarify accountability for safe use and record validation.



ARE YOU ON TRACK? – BENEFITS AND WHAT’S IN IT FOR ME (WIIFM)

Pathway 1 - We are starting AVT for the first time Benefits and WIIFM

Descriptor: Organisations are defining the benefits that matter for staff, patients and services.

Are you on track?

- ☐ Define clear, realistic benefits for staff, patients and services before rollout begins.
- ☐ Prioritise the benefits that matter most for the pilot setting and use case.
- ☐ Link expected benefits directly to the case for change and vision for AVT.
- ☐ Agree how benefits will be measured using both qualitative and quantitative evidence.
- ☐ Set realistic expectations about when benefits are likely to emerge and where they may be limited.



ARE YOU ON TRACK? – BENEFITS AND WHAT’S IN IT FOR ME (WIIFM)

Pathway 2 - We have piloted AVT and need to scale up Benefits and WIIFM

Descriptor: Organisations are using real evidence to strengthen and spread the benefits case.

Are you on track?

- ☐ Capture real benefit evidence from early adopters and pilot services.
- ☐ Compare expected benefits against actual experience across different teams and settings.
- ☐ Share examples of realised benefits in a way that is meaningful to different staff groups.
- ☐ Refine benefit assumptions where early rollout shows variation by workflow or specialty.
- ☐ Track both experience-based benefits and operational signals as rollout expands.
- ☐ Use benefit evidence to inform decisions about wider rollout, pace and targeting.
- ☐ Keep reinforcing the benefits that matter most to staff, not just the ones easiest to report.



ARE YOU ON TRACK? – BENEFITS AND WHAT’S IN IT FOR ME (WIIFM)

Pathway 3 - AVT is live but adoption is patchy Benefits and WIIFM

Descriptor: Organisations are rebuilding belief where benefits are unclear or not being realised.

Are you on track?

- ☐ Identify where expected benefits are not being realised in practice.
- ☐ Diagnose whether weak benefits are driven by adoption, workflow, capability or poor fit.
- ☐ Reset unrealistic or misunderstood expectations where needed.
- ☐ Refocus attention on benefits that matter to staff and patients in that setting.
- ☐ Use relevant local examples to rebuild belief where benefit claims have lost credibility.
- ☐ Stop relying on theoretical benefit claims that staff no longer recognise as real.





ARE YOU ON TRACK? – STAKEHOLDER ENGAGEMENT AND COMMUNICATION

Pathway 1 - We are starting AVT for the first time Stakeholder Engagement and Communication

Descriptor: Organisations are building early awareness, trust and understanding.

Are you on track?

- ☐ Define clear, simple messages about what AVT is and what it is not.
- ☐ Explain why AVT is being introduced in terms staff care about.
- ☐ Identify key stakeholder groups across clinical, admin, operational and digital areas.
- ☐ Tailor messages to different roles and workflows.
- ☐ Identify early adopters and involve them early.
- ☐ Set realistic expectations about early challenges and learning curves.



ARE YOU ON TRACK? – STAKEHOLDER ENGAGEMENT AND COMMUNICATION

Pathway 2 - We have piloted AVT and need to scale up Stakeholder Engagement and Communication

Descriptor: Organisations are reinforcing messaging and maintaining consistency across teams.

Are you on track?

- ☐ Ensure communication is consistent across services and teams.
- ☐ Reinforce messages through leaders, champions and everyday conversations.
- ☐ Share real examples of AVT use in practice.
- ☐ Communicate how AVT is improving care and experience locally.
- ☐ Challenge misinformation and unhelpful narratives.
- ☐ Keep feedback channels visible and accessible.
- ☐ Use feedback to adapt messaging and address emerging concerns.
- ☐ Re-engage teams where rollout has been slower or more complex.



ARE YOU ON TRACK? – STAKEHOLDER ENGAGEMENT AND COMMUNICATION

Pathway 3 - AVT is live but adoption is patchy Stakeholder Engagement and Communication

Descriptor: Organisations are addressing confusion, disengagement and mixed messages.

Are you on track?

- ☐ Identify where communication has become unclear or inconsistent.
- ☐ Map which teams or roles are disengaged or poorly informed.
- ☐ Address specific concerns openly, including trust, accuracy and workflow impact.
- ☐ Use peer-led conversations to rebuild credibility.
- ☐ Share relevant examples from comparable teams.
- ☐ Re-establish regular communication where it has dropped away.



ARE YOU ON TRACK? – PROCESS CHANGE AND WAYS OF WORKING

Pathway 1 - We are starting AVT for the first time Process Change and Ways of Working

Descriptor: Organisations are designing AVT-enabled workflows across clinical and administrative practice.

Are you on track?

- ☐ Map current clinical and administrative workflows before introducing AVT.
- ☐ Identify where documentation burdens, delays and handoff issues currently occur.
- ☐ Define future workflows that incorporate AVT into real operational practice.
- ☐ Involve clinicians, admin staff and operational teams in designing new workflows.
- ☐ Define how AVT fits into consultations, documentation and follow-up processes.
- ☐ Design how clinical and admin handoffs will work with AVT in place.
- ☐ Identify where workflows differ by service or specialty.
- ☐ Test new workflows in practice and refine them before wider rollout.
- ☐ Ensure operational leaders are involved in agreeing and owning new ways of working.





ARE YOU ON TRACK? – PROCESS CHANGE AND WAYS OF WORKING

Pathway 2 - We have piloted AVT and need to scale up Process Change and Ways of Working

Descriptor: Organisations are embedding and standardising AVT-enabled workflows across services.

Are you on track?

- ☐ Define clear and repeatable workflows for AVT-enabled practice.
- ☐ Update SOPs and operational guidance to reflect those workflows.
- ☐ Clarify roles and responsibilities across clinical and admin teams.
- ☐ Standardise key handoffs while allowing for local service differences.
- ☐ Manage variation actively rather than letting teams evolve ad hoc practices.
- ☐ Monitor where workflows are breaking down or not being followed.
- ☐ Address behavioural and operational barriers to adopting new processes.
- ☐ Support services to adapt workflows locally while maintaining core standards.



ARE YOU ON TRACK? – PROCESS CHANGE AND WAYS OF WORKING

Pathway 3 - AVT is live but adoption is patchy Process Change and Ways of Working

Descriptor: Organisations are fixing broken workflows and removing workarounds.

Are you on track?

- ☐ Identify where teams are not following the intended workflow.
- ☐ Spot where staff are reverting to old processes or workarounds.
- ☐ Pinpoint the specific parts of the workflow that are failing.
- ☐ Retire old processes that are still running in parallel.
- ☐ Reinforce correct workflows through local leadership and support.
- ☐ Remove practical barriers preventing teams from using AVT properly.



ARE YOU ON TRACK? – CHANGE IMPACT AND RISK

Pathway 1 - We are starting AVT for the first time Change Impact and Risk

Descriptor: Organisations are understanding who is impacted and where risk sits before rollout.

Are you on track?

- ☐ Identify all roles impacted by AVT across clinical, admin and operational areas.
- ☐ Define how workflows, responsibilities and behaviours will change for each role.
- ☐ Assess where impact differs across specialties and services.
- ☐ Identify high-risk use cases and workflows early.
- ☐ Engage impacted staff to validate and refine identified impacts.
- ☐ Assess risks relating to safety, accuracy and workflow disruption.
- ☐ Define how key risks will be monitored and mitigated during the pilot.



ARE YOU ON TRACK? – CHANGE IMPACT AND RISK

Pathway 2 - We have piloted AVT and need to scale up Change Impact and Risk

Descriptor: Organisations are monitoring real-world impact and managing emerging risks.

Are you on track?

- ☐ Monitor how impacts vary across teams and settings as rollout expands.
- ☐ Identify where changes are not delivering expected outcomes.
- ☐ Track risks emerging from real use, including workarounds and delays.
- ☐ Gather structured feedback from impacted staff groups.
- ☐ Adjust workflows and expectations based on real-world impact.
- ☐ Review impact and risk regularly through governance routes.



ARE YOU ON TRACK? – CHANGE IMPACT AND RISK

Pathway 3 - AVT is live but adoption is patchy Change Impact and Risk

Descriptor: Organisations are addressing the impacts and risks that are limiting adoption.

Are you on track?

- ☐ Identify where AVT is creating unintended burden, frustration or risk.
- ☐ Pinpoint the roles or teams most negatively impacted.
- ☐ Distinguish between real risks, perceived risks and misunderstood risks.
- ☐ Address the specific issues preventing safe and confident use.
- ☐ Focus support on the highest-risk and most impacted areas.



ARE YOU ON TRACK? – SKILLS, LEARNING, CAPABILITY AND CONFIDENCE

Pathway 1 - We are starting AVT for the first time Skills, Learning, Capability and Confidence

Descriptor: Organisations are introducing role-based learning and practical support.

Are you on track?

- ☐ Design learning around real clinical and administrative workflows rather than system features alone.
- ☐ Show staff how AVT fits their role, tasks and day-to-day practice.
- ☐ Provide practical hands-on opportunities to try AVT in relevant scenarios.
- ☐ Make support available during early live use, not just before go-live.
- ☐ Set expectations that confidence will build through use and repetition.
- ☐ Explain what good use looks like in practice for different roles and settings.



ARE YOU ON TRACK? – SKILLS, LEARNING, CAPABILITY AND CONFIDENCE

Pathway 2 - We have piloted AVT and need to scale up Skills, Learning, Capability and Confidence

Descriptor: Organisations are reinforcing learning through real use and ongoing support.

Are you on track?

- ☐ Reinforce learning through repetition and real-world use across services.
- ☐ Encourage peer-to-peer learning and sharing of practical tips between teams.
- ☐ Provide support at the point of need during real workflows and live sessions.
- ☐ Update learning materials based on user feedback and common issues.
- ☐ Build local capability within teams so support is not dependent on one person or one central team.
- ☐ Monitor confidence across different professions, specialties and settings.
- ☐ Refresh learning and support when rollout expands into new services or contexts.
- ☐ Revisit support after breaks, rota changes or service moves so learning is sustained.



ARE YOU ON TRACK? – SKILLS, LEARNING, CAPABILITY AND CONFIDENCE

Pathway 3 - AVT is live but adoption is patchy Skills, Learning, Capability and Confidence

Descriptor: Organisations are closing capability gaps that are preventing confident use.

Are you on track?

- ☐ Identify where staff lack confidence or practical capability to use AVT well.
- ☐ Target support to the roles, teams or services where capability gaps are most visible.
- ☐ Deliver support in the workflow rather than relying on more classroom-style training.
- ☐ Use local super users to model and reinforce good practice in real settings.
- ☐ Address the specific capability gaps that are preventing consistent use.



ARE YOU ON TRACK? – CHANGE ADOPTION

Pathway 1 - We are starting AVT for the first time Change Adoption

Descriptor: Organisations are defining what good adoption looks like from the start.

Are you on track?

- ☐ Define what successful adoption will look like for the pilot and who it will apply to.
- ☐ Identify which teams, roles or use cases are most likely to adopt early.
- ☐ Establish how adoption will be monitored using both usage data and staff feedback.
- ☐ Set expectations that adoption will vary in the early stages.
- ☐ Put practical support mechanisms in place from day one.
- ☐ Define what would count as a meaningful early sign of adoption in practice.



ARE YOU ON TRACK? – CHANGE ADOPTION

Pathway 2 - We have piloted AVT and need to scale up Change Adoption

Descriptor: Organisations are monitoring, guiding and strengthening adoption at scale.

Are you on track?

- ☐ Monitor adoption across teams, roles and services as rollout expands.
- ☐ Identify variation in how AVT is being used across specialties and settings.
- ☐ Target support to areas with lower confidence, lower uptake or inconsistent use.
- ☐ Reinforce expected AVT behaviours through leaders, champions and practice-based support.
- ☐ Use both data and frontline insight to guide rollout decisions and adjustments.
- ☐ Distinguish between access, trial, routine use and selective use when reviewing adoption.
- ☐ Adjust the rollout approach where adoption patterns show mismatch between the tool and the workflow.
- ☐ Keep reinforcing what good adoption looks like as more teams come on board.



ARE YOU ON TRACK? – CHANGE ADOPTION

Pathway 3 - AVT is live but adoption is patchy Change Adoption

Descriptor: Organisations are identifying why use is inconsistent and taking action.

Are you on track?

- ☐ Identify where adoption is inconsistent, superficial or declining.
- ☐ Investigate the root causes of selective use, workarounds or avoidance.
- ☐ Address specific behaviours or barriers that are undermining confident routine use.
- ☐ Reset expectations through targeted frontline engagement and support.
- ☐ Use local leaders and peers to reinforce intended use where credibility is low.
- ☐ Reassess whether AVT is being used in services where it genuinely fits the workflow.



ARE YOU ON TRACK? – TRANSFER TO BUSINESS AS USUAL (BAU)

Pathway 1 - We are starting AVT for the first time Transfer to BAU

Descriptor: Organisations are planning how AVT will be supported and owned beyond rollout.

Are you on track?

- ☐ Define who will own AVT after the pilot and what that ownership includes.
- ☐ Plan how AVT will be supported operationally once initial rollout activity reduces.
- ☐ Define how AVT will be reviewed, improved and maintained beyond go-live.
- ☐ Identify the governance routes AVT will sit within once it becomes business as usual.
- ☐ Clarify how issues, template changes and workflow changes will be managed over time.
- ☐ Decide how benefits, adoption and risk will continue to be monitored after rollout.



ARE YOU ON TRACK? – TRANSFER TO BUSINESS AS USUAL (BAU)

Pathway 2 - We have piloted AVT and need to scale up Transfer to BAU

Descriptor: Organisations are embedding AVT into business-as-usual governance and support.

Are you on track?

- ☐ Confirm ownership of AVT at organisational, service and operational level as rollout expands.
- ☐ Embed AVT into routine governance, reporting and operational review forums.
- ☐ Ensure support routes are clear and accessible across all services using AVT.
- ☐ Review and refine templates, workflows and guidance as AVT use spreads.
- ☐ Capture learning from early adopters and use it to strengthen BAU arrangements.
- ☐ Align onboarding, induction and refresher support so new users can adopt AVT reliably.
- ☐ Ensure BAU arrangements support sustained use, not just technical availability.



ARE YOU ON TRACK? – TRANSFER TO BUSINESS AS USUAL (BAU)

Pathway 3 - AVT is live but adoption is patchy Transfer to BAU

Descriptor: Organisations are re-establishing AVT as routine operational practice.

Are you on track?

- ☐ Identify where AVT has drifted out of routine use or lost operational ownership.
- ☐ Reconfirm who is responsible for adoption, support and improvement at service level.
- ☐ Fix gaps in support routes where staff do not know how to get help.
- ☐ Re-embed AVT into normal governance and operational forums if necessary.
- ☐ Reset BAU expectations where AVT is still being treated as a temporary project tool.



ARE YOU ON TRACK? – CHANGE NETWORK

Pathway 1 - We are starting AVT for the first time Change Network

Descriptor: Organisations are establishing a credible network of local champions and supporters.

Are you on track?

- ☐ Identify credible local champions who are respected by peers and close to frontline work.
- ☐ Define what the change network is there to do beyond simple promotion of AVT.
- ☐ Involve champions early in testing messages, workflows and early use cases.
- ☐ Ensure the network includes relevant clinical, admin and operational voices where possible.
- ☐ Give champions the information and support they need to answer questions credibly.
- ☐ Make it clear how champions will feed insight back into the programme.



ARE YOU ON TRACK? – CHANGE NETWORK

Pathway 2 - We have piloted AVT and need to scale up Change Network

Descriptor: Organisations are using the network to spread learning and support local adoption.

Are you on track?

- ☐ Use the change network to share practical examples of AVT use across services.
- ☐ Support champions to help teams adapt AVT to local workflows.
- ☐ Keep the network active through regular contact, shared learning and visible support.
- ☐ Use champions to surface emerging issues early rather than waiting for formal escalation.
- ☐ Expand the network as rollout spreads into new areas and specialties.
- ☐ Avoid over-relying on one or two enthusiastic individuals by broadening local capability.
- ☐ Keep the network connected to clinical leadership and programme decision-making.



ARE YOU ON TRACK? – CHANGE NETWORK

Pathway 3 - AVT is live but adoption is patchy Change Network

Descriptor: Organisations are reactivating the network to support struggling teams.

Are you on track?

- ☐ Identify where or if there is no credible local championing or peer support.
- ☐ Re-engage trusted local people who can model AVT use in practice.
- ☐ Use the network to understand why specific teams have disengaged or stalled.
- ☐ Use peer influence to rebuild credibility where central messaging is no longer landing.
- ☐ Reconnect the network to practical support and local problem-solving where it has drifted into inactivity.





ARE YOU ON TRACK? – REINFORCEMENT

Pathway 1 - We are starting AVT for the first time Reinforcement

Descriptor: Organisations are reinforcing early use through support, feedback and visible examples

Are you on track?

- ☐ Define how AVT use will be reinforced during the early learning period.
- ☐ Reinforce the purpose of AVT through repeated messages from leaders and champions.
- ☐ Build in opportunities for staff to review, correct and learn from early AVT outputs.
- ☐ Set clear expectations that early friction is normal and support will be available.
- ☐ Use visible early examples of success to strengthen confidence and belief.
- ☐ Decide how feedback from early use will be acted on and communicated back.





ARE YOU ON TRACK? – REINFORCEMENT

Pathway 2 - We have piloted AVT and need to scale up Reinforcement

Descriptor: Organisations are reinforcing confident and routine use in practice.

Are you on track?

- ☐ Reinforce AVT use through visible peer practice and trusted local champions.
- ☐ Keep support available at the point of use as rollout widens.
- ☐ Share practical examples of how teams are making AVT work in real settings.
- ☐ Show clearly what has changed in response to staff feedback.
- ☐ Reinforce the personal and workflow benefits staff are actually experiencing.
- ☐ Protect teams through the early dip in confidence rather than judging success too soon.
- ☐ Keep repeating the core purpose, expected use and support routes as rollout expands.



ARE YOU ON TRACK? – REINFORCEMENT

Pathway 3 - AVT is live but adoption is patchy Reinforcement

Descriptor: Organisations are restoring why AVT is important where usage has dropped and support is not there.

Are you on track?

- ☐ Identify where AVT usage has dropped away or workarounds have appeared.
- ☐ Reintroduce local support, peer-to-peer support and visible leadership where use has drifted.
- ☐ Use practical examples and feedback loops to rebuild confidence and routine use.
- ☐ Address the specific issues teams are encountering rather than repeating generic rollout messages.
- ☐ Check whether the reinforcement strategy is actually changing behaviour or just increasing awareness.



ARE YOU ON TRACK? – RESISTANCE MANAGEMENT

Pathway 1 - We are starting AVT for the first time Resistance

Descriptor: Organisations are identifying and addressing likely concerns before rollout.

Are you on track?

- ☐ Identify the main concerns staff are likely to have about AVT before rollout begins.
- ☐ Distinguish between legitimate concerns, misconceptions and assumptions about the technology.
- ☐ Address likely issues such as accuracy, workflow fit, trust, consent and accountability early.
- ☐ Give staff permission to voice concerns without being labelled resistant.
- ☐ Build realistic expectations about what AVT can and cannot do at this stage.
- ☐ Explain where AVT may not fit every setting perfectly and why professional judgement still matters.



ARE YOU ON TRACK? – RESISTANCE MANAGEMENT

Pathway 2 - We have piloted AVT and need to scale up Resistance

Descriptor: Organisations are managing emerging resistance as rollout expands.

Are you on track?

- ☐ Monitor where resistance is emerging across teams, roles or specialties.
- ☐ Identify whether resistance is being driven by workflow, trust, capability or poor fit.
- ☐ Respond to concerns with practical answers, not generic reassurance.
- ☐ Use local peers and credible examples to reduce anxiety and skepticism.
- ☐ Avoid treating superficial or selective use as successful adoption.
- ☐ Track whether resistance is reducing, shifting or becoming more hidden as rollout progresses.
- ☐ Adjust rollout pace or expectations where resistance reflects real limitations of the setting.



ARE YOU ON TRACK? – RESISTANCE MANAGEMENT

Pathway 3 - AVT is live but adoption is patchy Resistance

Descriptor: Organisations are identifying and resolving embedded resistance and workarounds.

Are you on track?

- ☐ Identify where resistance is active, hidden or showing up as workarounds and selective use.
- ☐ Find out what is driving that resistance in the specific team or setting.
- ☐ Address concerns through practical changes, visible support and credible local discussion.
- ☐ Stop treating all low adoption as poor attitude when it may reflect real workflow or trust issues.
- ☐ Reset expectations or usage where the current implementation does not fit the setting well.



ARE YOU ON TRACK? – READINESS AND PLANNING

Pathway 1 - We are starting AVT for the first time Readiness and Planning

Descriptor: Organisations are preparing and assessing readiness for a safe and credible pilot.

Are you on track?

- ☐ Assess organisational readiness for AVT across clinical, operational, digital and governance areas.
- ☐ Identify the minimum conditions needed for a safe and credible pilot to begin.
- ☐ Confirm readiness of core enablers such as devices, connectivity, workflow design and support arrangements.
- ☐ Clarify the scope, objectives and success criteria for the pilot before launch.
- ☐ Define the implementation plan, roles, milestones and decision points for the pilot phase.
- ☐ Sequence preparatory activity so governance, communications, training and workflow changes are in place before go-live.
- ☐ Check whether the service, team or setting is genuinely ready to pilot rather than proceeding on programme pressure alone.





ARE YOU ON TRACK? – READINESS AND PLANNING

Pathway 2 - We have piloted AVT and need to scale up Readiness and Planning

Descriptor: Organisations are planning rollout based on readiness, not just pace.

Are you on track?

- ☐ Review whether services are genuinely ready to scale rather than assuming pilot success will transfer automatically.
- ☐ Plan rollout in stages based on readiness, workflow fit and local capability.
- ☐ Confirm that each new service has the required governance, support, training and technical setup in place before expansion.
- ☐ Use readiness checks to identify where rollout should proceed, pause or be adapted.
- ☐ Align implementation plans across services while allowing for local workflow differences.
- ☐ Build review points into the scaling plan so learning from early rollout can inform later phases.
- ☐ Ensure planning includes realistic capacity for support, championing and problem-solving as scale increases.
- ☐ Avoid treating readiness as a one-off by checking it repeatedly as rollout moves into new areas.



ARE YOU ON TRACK? – READINESS AND PLANNING

Pathway 3 - AVT is live but adoption is patchy Readiness and Planning

Descriptor: Organisations are correcting issues following rollout.

Are you on track?

- ☐ Identify whether patchy adoption reflects poor readiness, weak planning or both.
- ☐ Review whether the original rollout sequence moved too quickly or into services that were not ready.
- ☐ Pinpoint which missing conditions are now undermining adoption (for example support, workflow fit, governance clarity or infrastructure issues).
- ☐ Reset the implementation plan for affected teams rather than continuing to push rollout at pace.
- ☐ Re-establish realistic next steps, ownership and timing for recovering adoption in those areas.



ARE YOU ON TRACK? – ORGANISATIONAL CHANGE MANAGEMENT MATURITY

Pathway 1 - We are starting AVT for the first time Organisational Change Management Maturity

Descriptor: Organisations are identifying and addressing likely concerns before rollout.

Are you on track?

- ☐ Identify the main concerns staff are likely to have about AVT before rollout begins.
- ☐ Distinguish between legitimate concerns, misconceptions and assumptions about the technology.
- ☐ Address likely issues such as accuracy, workflow fit, trust, consent and accountability early.
- ☐ Give staff permission to voice concerns without being labelled resistant.
- ☐ Build realistic expectations about what AVT can and cannot do at this stage.
- ☐ Explain where AVT may not fit every setting perfectly and why professional judgement still matters.



ARE YOU ON TRACK? – ORGANISATIONAL CHANGE MANAGEMENT MATURITY

Pathway 2 - We have piloted AVT and need to scale up Organisational Change Management Maturity

Descriptor: Organisations are strengthening change capability to support wider rollout.

Are you on track?

- ☐ Review whether change management maturity is strong enough to support rollout beyond early adopters.
- ☐ Identify where maturity varies across services, teams or professional groups as rollout expands.
- ☐ Strengthen weak areas such as local leadership, communication, reinforcement and support before scaling further.
- ☐ Use maturity insight to plan where scale can proceed and where additional change support is needed.
- ☐ Ensure change management approaches are consistent enough to support rollout while still allowing local flexibility.
- ☐ Build change capability within services so adoption does not rely only on a central programme team.
- ☐ Reassess maturity at key scale points rather than treating it as a one-off diagnostic.
- ☐ Use maturity findings to target practical improvement activity, not just create a status report.





ARE YOU ON TRACK? – ORGANISATIONAL CHANGE MANAGEMENT MATURITY

Pathway 3 - AVT is live but adoption is patchy Organisational Change Management Maturity

Descriptor: Organisations are addressing maturity gaps that are causing inconsistent adoption.

Are you on track?

- ☐ Identify whether patchy adoption reflects weaknesses in change management maturity rather than the technology alone.
- ☐ Pinpoint where weak sponsorship, communications, local ownership or reinforcement are contributing to low adoption.
- ☐ Use maturity insight to focus recovery activity on the weakest change mechanisms in affected teams.
- ☐ Strengthen practical change disciplines where organisations are relying on access to AVT without enough support to embed it.
- ☐ Reassess maturity after corrective action to test whether the organisation is better placed to sustain adoption.

ADDITIONAL INFORMATION

NHS England » Guidance on the use of AI-enabled ambient scribing products in health and care settings

NHS England Change Management Toolkit

Further Resources